Northmarq

FOURTH QUARTER 2025 UPDATE | DATA ENDING DECEMBER 31, 2025

The Dialysis Market FILTER

Covering Trends in the Single-Tenant Outpatient Dialysis Clinic Real Estate Investment Sales Market

Dear Clients and Partners,

We maintain this report daily and update it periodically to track trends and trades—at a broad-market level—in the single-tenant office, medical, and dialysis-leased investment sales markets.

As part of Northmarq's National Healthcare Group (NHG), our dialysis team continues to lead the single-tenant dialysis vertical nationwide. Our healthcare practice has completed more than 810 healthcare transactions totaling $4.37 billion, including over 270 dialysis clinic sales. Our current pipeline of exclusive listings and confidential offerings remains active across the U.S., reflecting durable investor demand for essential healthcare cash flows.

Beyond volume, our execution consistently delivers a measurable pricing advantage for sellers. Over the trailing twelve months, marketed dialysis assets placed by Northmarq have achieved an average cap rate of **6.70%** versus **7.33%** for non-Northmarq transactions—translating to approximately **$265,788** in additional proceeds per sale **(+10% uplift)** and an average sales price of **$3.06 million** versus **$2.80 million** outside our platform.

The pages that follow are intended to deliver real-time value to you and your business and to demonstrate the level of service and quality our team brings to every assignment. If there's anything we can do to assist—valuation, disposition strategy, capital sourcing, or market intelligence—please reach out. We look forward to leveraging our specialization, knowledge, track record, and execution to your benefit.

Best Regards,

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What's New in Q4 2025

+11.6%
Year-Over-Year

TRANSACTION VOLUME RECOVERY

+11.6% YoY, signaling renewed buyer confidence after the 2023–2024 slowdown.

7.57%
TTM Average

CAP RATE STABILIZATION

TTM average 7.57% (+25 bps annualized vs. +39 bps at mid-year), with pricing momentum improving.

4.12%
10-Year UST

RATE ENVIRONMENT EASING

10Y UST at 4.12% (−27 bps from mid-year); Fed Funds at 4.33% following consecutive cuts.

$5.3 Trillion
18.0% of GDP

NHE MILESTONE

U.S. healthcare spending reached $5.3 trillion in 2024 (18.0% of GDP), reinforcing structural demand for essential healthcare real estate.

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Table of Contents

SECTION ONE | DIALYSIS INDUSTRY
Defining the Dialysis Industry . . . 6
Dialysis Industry FAQ . . . 7
Anatomy of the Kidney Shortage & Major Players . . . 8
Dialysis as a Commercial Real Estate Occupant . . . 9
Industry Participants . . . 10
Standard Build-to-Suit Lease Terms Comparison . . . 11
Dialysis Market News . . . 12
Ozempic and the Dialysis Industry . . . 13
Advancing American Kidney Health Initiative . . . 14

SECTION TWO | DIALYSIS CAPITAL MARKETS
Valuation Index . . . 17
Annualized Cash and Leveraged Return . . . 18
Volume & Cap Rate Ranges . . . 19
Trend Watch . . . 20
Transaction Velocity & Allocation to Single-Tenant Dialysis . . . 21
Cap Rate Comparison: The Value of Lease Term . . . 22
Cost of Capital & Interest Rate Impact on Valuation . . . 23
Pace of Cap Rate Expansion . . . 24
Transaction Count & Average Deal Size . . . 25
Rent & Price Per Square Foot . . . 26
Buyer Pool & Cap Rates . . . 27

SECTION THREE | DIALYSIS INVENTORY ANALYSIS
On-Market Snapshot . . . 29
Supply Side Metrics . . . 30
Available Market Size . . . 31
Available Clinics by Tenant . . . 32
Days on Market & Percentage of Ask Price . . . 33
Bid-Ask Spread . . . 34
Supply Side: Seller Sentiment and Confidence . . . 35

SECTION FOUR | ABOUT NORTHMARQ
Yielding Results . . . 37
Value Proposition Results . . . 38
Healthcare Track Record . . . 39
Sample Dialysis Transactions . . . 40
Services Offered . . . 41
Northmarq Debt + Equity . . . 42

Acronym Glossary . . . 43

Section One

DIALYSIS INDUSTRY

Defining the Dialysis Industry

DIALYSIS, KIDNEY CARE AND THE HEALTHCARE INDUSTRY

U.S. healthcare spending continues its steady, long-term rise—driven by demographics (aging baby boomers), lifestyle factors, advancing medical technologies and pharmaceuticals, and reimbursement structures. In 2024, CMS reported national health expenditures grew 7.2% to $5.3 trillion, accounting for 18.0% of U.S. GDP (up from 17.7% in 2023). Per capita health spending reached $15,474. CMS projects health spending growth of 7.1% in 2025 and expects the health share of GDP to reach 20.3% by 2033. Medicare spending increased 7.8%, to $1.12 trillion in 2024, with the CBO projecting net Medicare outlays could reach $3.3 trillion (≈8.3% of GDP) by 2033. ***These trends reinforce structural demand for essential kidney care services and the outpatient dialysis real estate footprint.***

INDUSTRY FAST FACTS

- **808,000** ESRD dialysis patients in the U.S. in 2023[1]; **3.3%** CAGR from 2012-2022[2]
- There were **131,194** newly reported cases of end-stage renal disease in 2022[2]
- Total Medicare fee-for-service spending in the general Medicare population **grew 7.8%** in 2024 to **$1.12 trillion**[3]
- Kidney disease is the **9th leading cause of death** in the U.S.
- **1 in 7** U.S. adults has chronic kidney disease, most are not aware[1]
- According to the NIDDK, more than **one in five** Medicare dollars goes towards Kidney Care, totaling **±$129B** in 2021[1]
- There are currently nearly **89,800** people waiting for lifesaving kidney transplants in the U.S.[4] **85% are awaiting their 1st** kidney transplant

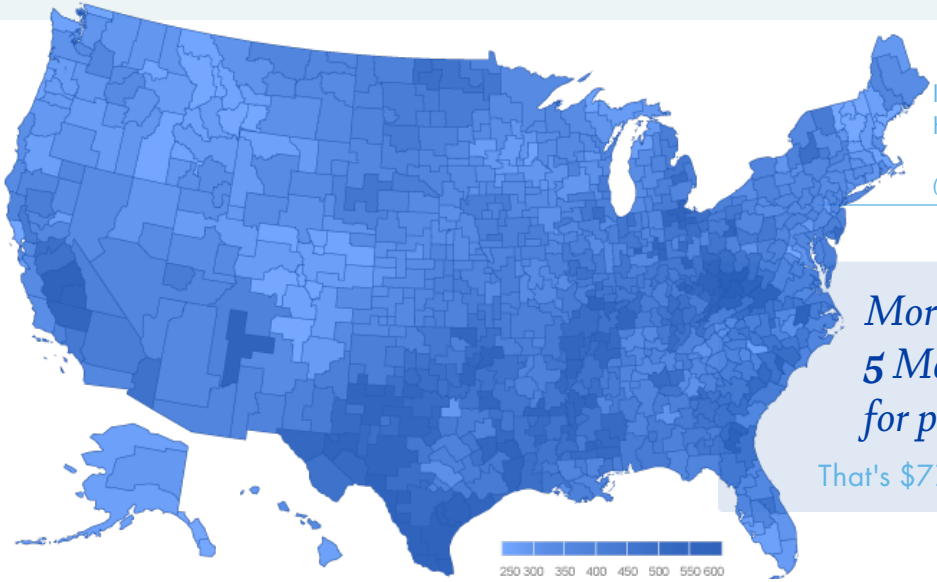
STAGE 1
Kidney Function >90%
Damage but normal kidney function; usually no symptoms.

STAGE 2
Kidney Function 60-89%
Mild loss; may see blood in urine or imaging-visible damage.

STAGE 3A & 3B
Kidney Function 45-59% & 30-44%
Moderate loss; fatigue, shortness of breath, ankle swelling

STAGE 4
Kidney Function 15-29%
Severe loss; dialysis/transplant likely in near future; symptoms may worsen

STAGE 5
Kidney Function <15%
Kidney failure (ESRD); dialysis and/or transplant required to live



INCIDENCE OF ESRD BY HEALTH SERVICE AREA (2024)

(Darker areas indicate higher incidence rate)[6]

Sources:
1 - NIH-NIDDK Kidney Disease Statistics
2 - NIH-NIDDK Annual Data Report
3 - CMS.gov NHE Fact Sheet
4 - HRSA - OrganDonor.gov Organ Donation Statistics
5 - U.S. Renal Data System 2024 Annual Report - Figure 1.3

*More than **1 out of every 5** Medicare dollars was for patients with CKD.*

That's $77 billion for people with CKD, and $52.3 billion for people with ESRD[1].

Dialysis Industry FAQ

WHAT IS CHRONIC KIDNEY DISEASE?

Chronic Kidney Disease (CKD) develops when the kidneys lose their ability to remove waste from and maintain fluid and chemical balances in the body. People with stage 5 CKD, or end-stage renal disease, require a transplant or dialysis for survival.

WHO IS AT RISK FOR CKD?

Higher-risk groups include African Americans, Hispanics, Pacific Islanders, Native Americans, and seniors (60+). Key risk factors: diabetes, hypertension, cardiovascular disease, and family history.

WHAT IS DIALYSIS?

Dialysis replaces kidney function by removing waste, salt, and excess fluid while preserving needed substances and safe electrolyte levels.

HOW DOES DIALYSIS WORK?

There are two main types of kidney dialysis—peritoneal dialysis (PD) and hemodialysis (HD).

PERITONEAL DIALYSIS (PD):

Uses the peritoneum and a dialysate fluid cycled in/out via a PD catheter.

HEMODIALYSIS (HD):

Uses an external filter (dialyzer) with a machine to clean the blood; commonly performed in-center (daytime or nocturnal) or at home.

What about a kidney transplant?

A kidney transplant can be a preferred treatment option for patients with kidney failure, depending on age and comorbidities. For many who are still waiting—or not eligible—dialysis can adequately replace kidney function for years.

The Stats and Facts About Kidney Transplants

±90,000
Number of people on the kidney transplant list

3–5 Years
Average wait time in years for a kidney donor match

38,000
Number of 50 to 64-year-olds waiting for a new kidney

27,300
Number of kidney transplants in 2023

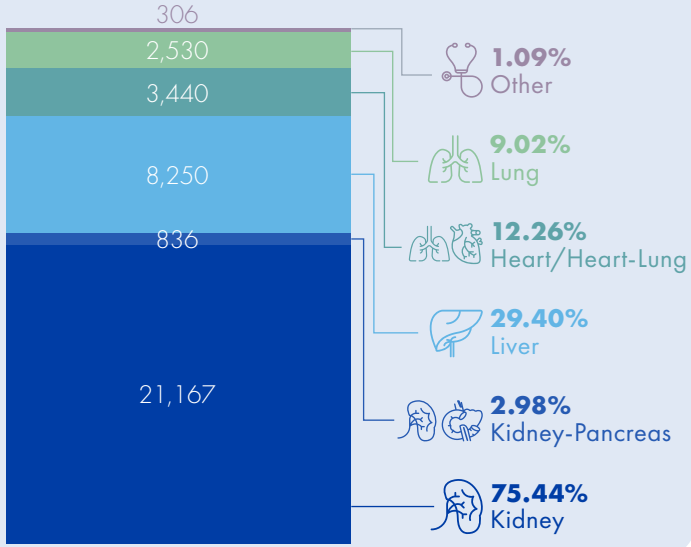
8 Minutes
A new patient is added to the kidney waiting list every 8 minutes

Sources:
https://www.organdonor.gov/learn/organ-donation-statistics
https://www.kidney.org/atoz/content/transplant-waitlist
https://optn.transplant.hrsa.gov/data/view-data-reports/national-data/#

Anatomy of the Kidney Shortage

Kidneys are unique among solid organs because living donation carries relatively low risk, patients can be maintained on dialysis for years, and Medicare covers dialysis and transplantation. Despite these advantages, **the waiting list grows each year** and thousands die waiting. Research indicates the system provides only about half the kidneys needed, and increasing deceased donation alone can't close the gap because most suitable deceased-donor kidneys are already procured. Despite Medicare coverage and the ability to sustain patients on dialysis for years, the U.S. still delivers only about half the kidneys needed each year, and the waitlist continues to grow; increasing deceased donation alone cannot close the gap because most suitable organs are already procured.

NUMBER OF TRANSPLANTS BY ORGAN, 2018



Major Players

COMPETITION IN THE OUTPATIENT DIALYSIS MARKET

The U.S. dialysis industry remains highly consolidated and highly competitive—especially around acquisitions of existing centers, physician medical-director relationships, and clinical talent.

Scale and share. Fresenius Medical Care ("FMC") and DaVita ("DVA") continue to dominate outpatient dialysis in the U.S., together accounting for roughly three-quarters to four-fifths of patients. Peer-reviewed research shows the combined share rose to ~77% by 2019, and recent industry analyses still place FMC ~37% with DVA in the high-30s, keeping the pair near that level today. As of Dec. 31, 2024, DaVita operated 3,166 outpatient centers (2,657 in the U.S.) serving ~281k patients globally; FMC reports ~3,675 clinics serving ~299k patients worldwide.

Mid-tier & independent operators. Regional and joint-venture models remain meaningful:

- **U.S. Renal Care** – cares for 36,000+ people across 32 states.
- **Dialysis Clinic, Inc. (DCI)** – the largest non-profit provider with 250+ clinics.
- **Innovative Renal Care** (ARA successor) – 250+ centers in 25 states via JV partnerships
- **Satellite Healthcare** – 90+ centers and an outsized focus on home dialysis programs.

Vertical integration & supply. FMC remains a vertically integrated kidney-care company (products + services), which can confer operating and sourcing advantages; FMC continues to be a major supplier of dialysis equipment and disposables across the market.

Reimbursement & revenue stability. While commercial plans remain important to facility margins, the vast majority of patients are covered by government programs (Medicare/Medicaid)—supporting predictable, recurring revenues tied to the ESRD bundle and annual updates monitored by MedPAC.

As a condition of enrollment in Medicare for the provision of dialysis services, participants contract with a nephrologist or a group of nephrologists to provide medical director services at each dialysis center. In addition, other nephrologists may apply for practice privileges to treat their patients at a clinic run by a market participant like DVA, FMC, or others. Each center has an administrator, typically a registered nurse, who supervises the day-to-day operations of the center and its staff. The staff typically consists of registered nurses, licensed practical or vocational nurses, patient care technicians, a social worker, a registered dietitian, biomedical technician support and other administrative and support personnel.

https://www.researchgate.net/publication/289419030_A_Primer_on_Kidney_Transplantation_Anatomy_of_the_Shortage

Dialysis as a Commercial Real Estate Occupant

SINGLE-TENANT TRENDS

Given the outstanding growth witnessed in the industry, a portion of outpatient dialysis centers are operating at or near capacity. However, participants can accommodate additional patient volume through increased hours and/or days of operation, or, if additional space is available within an existing facility, by adding dialysis stations. It is not the preference to increase shifts as a labor force is hard to find in most secondary markets. Outpatient clinic operators can usually relocate existing clinics to larger facilities or open new centers if existing centers reach capacity. Expansion of existing centers or relocation of dialysis centers is subject to review for compliance with conditions relating to participation in the Medicare ESRD program. In states that require a certificate of need or center license, additional approvals would generally be necessary for expansion or relocation.

We are spotting and witnessing a trend that is not unique to just the dialysis market but one which is pervasive in the healthcare industry's footprint: accessibility and convenience are becoming priorities and many of these clinics are beginning to leave major MOB campuses and mirror a single-tenant retail development with high frontage near area amenities such as restaurants and shopping. Many patients are dropped off for treatment by loved ones three days a week for their four-hour treatments and many participants have recognized a value-add for its client base by being near weekly shopping, restaurants and other area retail corridors.

As an example, according to our research there are currently 281 DaVita clinics in the state of Texas. Approximately 160 of these clinics are single-tenant or 57% of the total and just seven years ago this number was closer to 40% of the total. We estimate that this is higher than the historical trend and going forward we anticipate that both the retail and single-tenant trends coupled with the growing patient base will continue to cause the single-tenant outpatient dialysis clinic footprint to expand.

Industry Participants

	DaVita	FRESENIUS MEDICAL CARE
Corporate Entity:	DaVita, Inc. (DVA)	Fresenius Medical Care AG & Co. KGaA
Ticker:	NYSE: DVA	NYSE ADR: FMS FWB: FME
Headquarters:	Denver, Colorado	Bad Homburg vor der Höhe, Germany (U.S. HQ: Waltham, MA)
Employees:	±76,000 (FY 2024)	±111,500–112,000 (December 31, 2024)
Credit:	S&P: BB/Stable MIS: Ba2/Stable	S&P: BBB-/Stable MIS: Baa3/Stable FITCH: BBB-/Stable
U.S. Locations:	2,657 outpatient dialysis clinics in 46 states and DC (December 31, 2024)	2,800 outpatient dialysis clinics nationwide 3,675 clinics globally
Patients:	±200,800 dialysis patients (U.S.) (December 31, 2024)	±299,000 dialysis patients (Global) (December 31, 2024)
Reimbursement: (U.S. dialysis revenue mix, 2024)	Medicare & Medicare Advantage 56%; Medicaid & Managed Medicaid 8%; Other government 3%; Commercial (incl. hospital) 33%.	26% of revenue are covered under Medicare and Medicare-assigned plans
Recent Scale: (Global)	3,166 outpatient centers; ~281,100 patients (Dec 31, 2024).	
Vertical Integration:		FMC continues to be a leading supplier of dialysis machines, dialyzers, and disposables while operating clinics—an enduring procurement/operations advantage.
Market Share:	±36% of U.S. dialysis patients (2024)	±38%
Revenue:	$12.816B (FY 2024)	
Clinic Stats:	Range in size from approximately 600 to 33,000 square feet, with an average size of approximately 7,600 square feet Average 16.8 stations per clinic	Average 18.4 stations per clinic

Sources: DaVita Q4 2024 earnings release and 2024 10-K (filed February 2025); Fresenius Medical Care Q1 2025 results and FY 2025 guidance (published May 2025). Market-share estimates based on peer-reviewed literature (Crews et al., JASN 2019) and updated facility counts. Data as of dates shown.

OTHERS (REGIONAL & JOINT-VENTURE OPERATORS)

Independent and regional platforms remain material to local competition and JV growth:

U.S. Renal Care (USRC) – partners across 32 states, caring for 36,000+ people.

Dialysis Clinic, Inc. (DCI) – largest non-profit; 240+ outpatient clinics in 28–30 states.

Innovative Renal Care (IRC) – 250+ dialysis centers in ~25 states via JV partnerships (ARA successor platform).

Standard Build-to-Suit Lease Terms Comparison

	DaVita	FRESENIUS MEDICAL CARE	U.S. RENAL CARE
Tenant:	Total Renal Care, Inc. or DaVita affiliate	Bio-Medical Applications, Inc. or Fresenius affiliate	U.S. Renal Care affiliate
Guarantor:	DaVita, Inc. (NYSE: DVA)	Fresenius Medical Care Holdings, Inc.	U.S. Renal Care, Inc.
Credit:	Corporate	Wholly Owned Subsidiary	Corporate
Credit Rating (Aug '25):	S&P: BB / Ba2	S&P: BBB- / Baa3	S&P: B- (issue level)
Guaranty:	Initial term only	Initial and Option term	Initial and Option term
Initial Term:	Fifteen (15) Years	Fifteen (15) Years	Ten (10) Years
Early Termination:	None	None	None
Renewal Options:	Three (3), Five (5) Year	Two (2), Five (5) Year	Two (2), Five (5) Year
Increases:	Ten Percent (10%) Every Five (5)	Various	Two-and-a-Half Percent (2.5%) Annually
Expense Structure:	NN Plus Lease	NN Lease	Absolute NNN
Maintenance:	LL repair, replacement, and maintenance of roof and structure; replacement of HVAC and parking; T reimburses $1,000 per lease year for roof repairs; T reimburses the actual out of pocket costs plus carry of any replacement of roof, structure, parking, or HVAC amortized over the useful life of that item	LL repair, replacement, and maintenance of CAM, roof, structure, HVAC, parking, etc.; T reimburses CAM as additional rent	T maintains, repairs, and replaces all
Insurance:	T reimburses LL	T reimburses	T handles directly, names LL as additional insured
Taxes:	T pays directly	T reimburses	T pays directly
ROFR:	30 days	None	None

Dialysis Market News

Click any article below to read the full news story

FDA APPROVES OZEMPIC® TO REDUCE CKD PROGRESSION & CV DEATH IN T2D + CKD

January 28, 2025 – FDA added a kidney indication to Ozempic (semaglutide) after the FLOW outcomes trial; it's now the first GLP-1 RA approved to reduce risk of worsening kidney disease, kidney failure, and CV death in adults with type 2 diabetes and CKD.

Source: https://www.ozempic.com/content/dam/diabetes-patient/ozempic/pdfs/Ozempic_CKD_sNDA_Press_Release_January_28_2025.pdf

CMS FINALIZES MANDATORY IOTA TRANSPLANT MODEL (GO-LIVE JULY 1, 2025)

November 26, 2024 – CMS finalized the Increasing Organ Transplant Access (IOTA) model, a six-year mandatory program tying hospital payments to transplant performance, utilization, and equity metrics.

Source: https://www.cms.gov/newsroom/press-releases/biden-harris-administration-finalizes-new-model-improve-access-kidney-transplants

CY2025 ESRD PPS FINAL RULE: BASE RATE SET AT $273.82; CY2026 PROPOSED AT $281.06

November 12, 2024 / July 2025 – CMS finalized the 2025 ESRD PPS base rate at $273.82 (+1.0% vs. 2024) and later proposed $281.06 for 2026. Facilities should model modest revenue lift, subject to case-mix and wage-index adjustments.

Source: https://www.cms.gov/newsroom/fact-sheets/calendar-year-2025-end-stage-renal-disease-esrd-prospective-payment-system-pps-final-rule-cms-1805-f

FRESENIUS MEDICAL CARE: Q1 2025 ABOVE EXPECTATIONS; U.S. VOLUMES EXPECTED TO IMPROVE

May 6-11, 2025 – FMC beat Q1 consensus and reiterated 2025 outlook; management expects U.S. treatment growth to resume (>0.5% for 2025) as missed-treatment headwinds ease.

Source: https://www.reuters.com/business/healthcare-pharmaceuticals/fresenius-medical-care-tops-market-expectations-1st-quarter-2025-05-06/

DAVITA: Q2 2025 RESULTS SHOW MIXED VOLUME TRENDS BUT STEADY EXECUTION

August 5, 2025 – DaVita reported 7.19M U.S. treatments in Q2 (avg. 92,131/day), up 0.4% per-day Q/Q; normalized non-acquired treatment growth was -0.8% Year-Over-Year.

Source: https://investors.davita.com/2025-08-05-DaVita-Inc-2nd-Quarter-2025-Results

FDA APPROVES ORAL VAFSEO® (VADADUSTAT) FOR DIALYSIS-DEPENDENT ANEMIA

Source: Akebia, FDA

OUTPATIENT HEALTHCARE MOMENTUM CONTINUES; INVESTOR INTEREST IN MEDICAL REAL ESTATE STAYS STRONG

Source: JLL

Ozempic and the Dialysis Industry

Click any article below to read the full news story

FDA APPROVES OZEMPIC® TO REDUCE CKD PROGRESSION & CV DEATH IN T2D + CKD

January 28, 2025 – FDA added a kidney-protection indication to semaglutide (Ozempic) for adults with type 2 diabetes and chronic kidney disease, allowing labeled use to reduce risk of kidney disease worsening, kidney failure, and cardiovascular death.

Source: https://www.ozempic.com/content/dam/diabetes-patient/ozempic/pdfs/Ozempic_CKD_sNDA_Press_Release_January_28_2025.pdf

NEJM: FLOW TRIAL SHOWS 24% LOWER RISK OF KIDNEY/CV COMPOSITE OUTCOME

May 2024 – In 3,533 patients with T2D+CKD (median follow-up 3.4 years), weekly semaglutide cut the risk of the composite outcome (≥50% eGFR decline, kidney failure, or CV/kidney death) by 24% vs. placebo (HR 0.76; 95% CI 0.66–0.88).

Source: https://www.healio.com/news/endocrinology/20240305/flow-semaglutide-slows-ckd-progression-cuts-risks-for-kidney-cv-death-in-diabetes

WHAT THIS MEANS FOR DIALYSIS & NNN REAL ESTATE (2025 SNAPSHOT)

Near-term volumes remain resilient.
Providers continue to guide to modest U.S. treatment growth in 2025 (FMC: ">0.5%" same-market growth), and DaVita reported 7.19M U.S. treatments in Q2 2025 (92,131/day; +0.4% q/q; normalized non-acquired -0.8% y/y).

Scope of label matters.
The approval applies to T2D + CKD (not broadly to non-diabetic CKD), and benefits accrue over multiple years—tempering immediate impact on incident ESKD.

Watch reimbursement & adoption.
Real-world uptake depends on coverage, cost, and tolerability; clinics should monitor payer-mix and MA penetration alongside ESRD PPS updates (base rate $273.82 in CY2025; $281.06 proposed for CY2026).

Structural context.
The USRDS 2024 Annual Data Report confirms ESRD incidence declined 18.6% over the decade ending 2022—a trend that began well before GLP-1 RA adoption and reflects broader improvements in CKD screening, blood-pressure control, and SGLT2 inhibitor use. GLP-1 RAs (including semaglutide) add an additional, incremental tailwind to CKD progression delay. For NNN investors, this underscores that any volume impact from Ozempic will layer onto an already gradual incidence curve rather than creating a step-change disruption—and the growing prevalent ESRD population continues to support durable clinic demand.

Advancing American Kidney Health Initiative

On July 10th, 2019 President Trump signed and Executive Order titled "Advancing American Kidney Health Initiative." The original goals (2019 Executive Order):

- ***Reduce new ESKD cases by 25% by 2030***
- ***80% of new ESKD patients by 2025 to be on home dialysis or receive a transplant***
- ***Double the number of kidneys available for transplant by 2030.***

KEY POLICY LEVERS SINCE 2019

Mandatory ETC Model (CMS): Adjusts facility & nephrologist payments to encourage home dialysis and transplantation across selected HRRs (Model Years through mid-2027).

Voluntary KCC Model (CMS): Provider groups take accountability for late-stage CKD/ESRD outcomes (KCF/CKCC options; PY 2023–2026). Early CMS updates show quality gains (more optimal starts, higher home dialysis & living donor activity) but net Medicare losses in PY2023 (−$304M).

Lifetime immunosuppressive drug coverage (Part B-ID): Since Jan 1, 2023, eligible kidney recipients without other coverage can keep Medicare Part B immunosuppressive drug coverage indefinitely—aimed at graft longevity and fewer returns to dialysis.

Transplant system modernization: 2023's Securing the U.S. OPTN Act enabled HRSA to replace the single-vendor model with multiple contracts; HRSA's 2024 updates emphasize governance, performance, and IT upgrades.

Ongoing payment/quality updates: ESRD PPS base rate $273.82 for CY2025 (with additional policy tweaks); CY2026 update proposed. ESRD QIP continues to tie part of facility payment to quality.

WHERE THE GOALS STAND IN 2025 (WHAT'S ACTUALLY HAPPENING)

Home dialysis & transplant share are rising, but remain well short of the "80% by 2025" aspiration. USRDS 2024 ADR data show incident home modality use (PD and home HD) increased from 8.5% in 2012 to 14.5% in 2022. ETC evaluations show mixed or modest effects versus control HRRs; with the model running through mid-2027, gains are likely incremental rather than transformational.

ESKD incidence is trending down, with a potential new tailwind. USRDS 2024 ADR shows adjusted incidence fell ~18.6% through 2022, supporting AAKH's 2030 reduction goal.

Transplant access reforms are in motion. HRSA/OPTN modernization and hospital incentives (e.g., IOTA model you cite earlier) are the current levers to raise waitlisting and living-donor transplants—critical to the "double kidneys by 2030" aim.

WHAT TO WATCH NEXT

- ETC MY7–8 rules & benchmarks and any distributional effects on facilities with higher social risk.
- KCC PY2026: whether CMS modifies the model given PY2023 cost results.
- ESRD PPS CY2026 finalization timing and any case-mix/wage-index updates.
- OPTN multi-vendor awards and early performance/IT reliability milestones.

IMPLICATIONS FOR DIALYSIS NNN REAL ESTATE

Base demand remains durable (aging, diabetes prevalence), but mix shifts toward home modalities and higher transplant throughput argue for smarter clinic sizing and flexible layouts rather than wholesale volume contraction.

Operator strategy matters: Clinics with strong home programs, pre-ESKD care, and transplant partnerships should outperform on volumes/margins—supporting credit and lease durability.

Policy cadence (ETC/KCC/PPS/OPTN) introduces idiosyncratic market risk by region; investors should underwrite geographic selection and payer-mix with an eye on model benchmarks and local transplant performance. (Use your existing call-outs/sidebars to visualize these, try to bold or emphasize the text where it makes sense).

Advancing American Kidney Health Initiative

Fresenius Statement

U.S. PLANS FOR CHANGES TO KIDNEY DISEASE CARE AS POSITIVE REINFORCEMENT OF COMPANY'S STRATEGY

July 10, 2019 – Fresenius Medical Care, the world's largest provider of dialysis products and services, is pleased the U.S. administration's plans for changing the way care is provided to people with kidney disease supports its existing strategy. The company has long worked on various initiatives to promote home dialysis, improve access to transplants, and develop new, value-based care models for chronic kidney disease patients.

Rice Powell, CEO of Fresenius Medical Care, said: "We congratulate the Administration on today's announcement and celebrate the proposed initiatives as a win for our patients and for the 30 million Americans living with kidney disease. We are committed to continuously improving the quality of life of patients affected by kidney disease and have already established initiatives to improve prevention, offer more flexible treatment options, introduce value-based care models and promote organ donation –in the United States and abroad.

"We share the U.S. Administration's commitment to expanding access to home dialysis, transplantation and new models of value-based care for chronic kidney disease, and we see it as an endorsement of our initiatives. We invest constantly in innovation and will continue to do so in order to further develop the healthcare system. The proposed reimbursement models and new incentives will help foster further innovation and support a healthcare delivery system structure that is closely attuned to the needs of our patients.

"Our recent merger with NxStage, which makes the leading hemodialysis machine for home use, is just one piece of a focused effort to educate patients and physicians around the benefits of home treatment and provide industry leading solutions to enable them to do so." ***Read More »***

Source: https://www.freseniusmedicalcare.com/en/news/fresenius-medical-care-sees-us-plans-for-changes-to-kidney-disease-care-as-positive-reinforcement-of-companys-strategy

DaVita Statement

DAVITA CONTINUES TO TRANSFORM PATIENT CARE, PUSHES FOR CONTINUED INNOVATION IN KIDNEY CARE

July 9, 2019 – DaVita Kidney Care is energized by the focus from the Administration on kidney health.

These government initiatives are expected to support DaVita's investments to further kidney disease prevention, encourage home kidney care and improve kidney transplantation rates.

"We've been investing in capabilities to deliver holistic care that addresses our patients' needs beyond kidney disease, such as mental health, social services and nutrition. We will continue to work with the Administration and Congress to launch programs that address broader care opportunities," said Javier Rodriguez, CEO for DaVita Inc.

Read More »

Source: https://investors.davita.com/2019-07-09-DaVita-Continues-to-Transform-Patient-Care-Pushes-for-Continued-Innovation-in-Kidney-Care

Additional DaVita Statement

DAVITA STATEMENT ON ADVANCING AMERICAN KIDNEY HEALTH

July 10, 2019 – DaVita Kidney Care appreciates President Trump, the Administration and Secretary Azar for taking steps to improve the lives of kidney patients. We appreciate the Administration's willingness to collaborate with the industry to ensure these new models address patient choice, prevention and transplantation.

Read More »

Source: https://investors.davita.com/2019-07-10-DaVita-Statement-on-Advancing-American-Kidney-Health

Section Two

DIALYSIS CAPITAL MARKETS

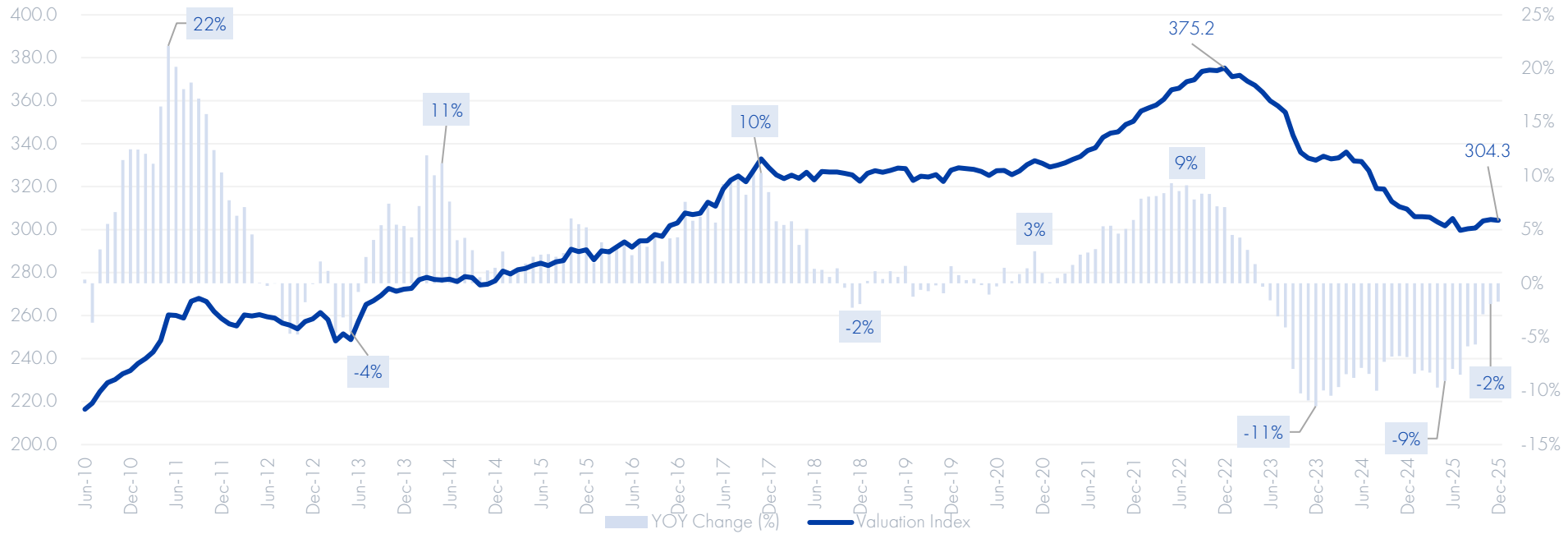
Dialysis Capital Markets

VALUATION INDEX

Pricing stabilization confirmed; modest easing underway. Underwriting for dialysis-leased assets completed its post-2022 repricing cycle through 2024, and the second half of 2025 delivered confirmation of stabilization as rate volatility continued to moderate and the 10-year U.S. Treasury drifted lower to ~4.12% on December 31, 2025. Within healthcare, outpatient demand remains a tailwind for income durability, with systems continuing to push care to ambulatory settings.

How to read the chart: The dark-blue line is our Dialysis Valuation Index—a nominal gauge that blends comparable cap-rate trends with changes in market rent and replacement cost to normalize values through cycles. The light-blue bars show YoY momentum in that index. Use the panel in tandem with the rate context (treasury trend) and market color from the following pages to calibrate expectations: (i) cap rates appear to have plateaued for most net-lease categories with selective compression beginning on higher-quality assets, (ii) lenders are quoting more actively than mid-2024, and (iii) medical/outpatient continues to draw capital as systems shift care from inpatient to ambulatory sites.

The fourth quarter ended with a valuation index of **304.3**. This is roughly **3% above** the fourteen-year average of 295.1 but **19% off** the all-time highs set in December 2022 of 375.2. Year-over-year, the index declined **1.7%**, a marked deceleration from the −8.0% read at mid-year and the −6.9% print a year ago—consistent with a market finding its floor.



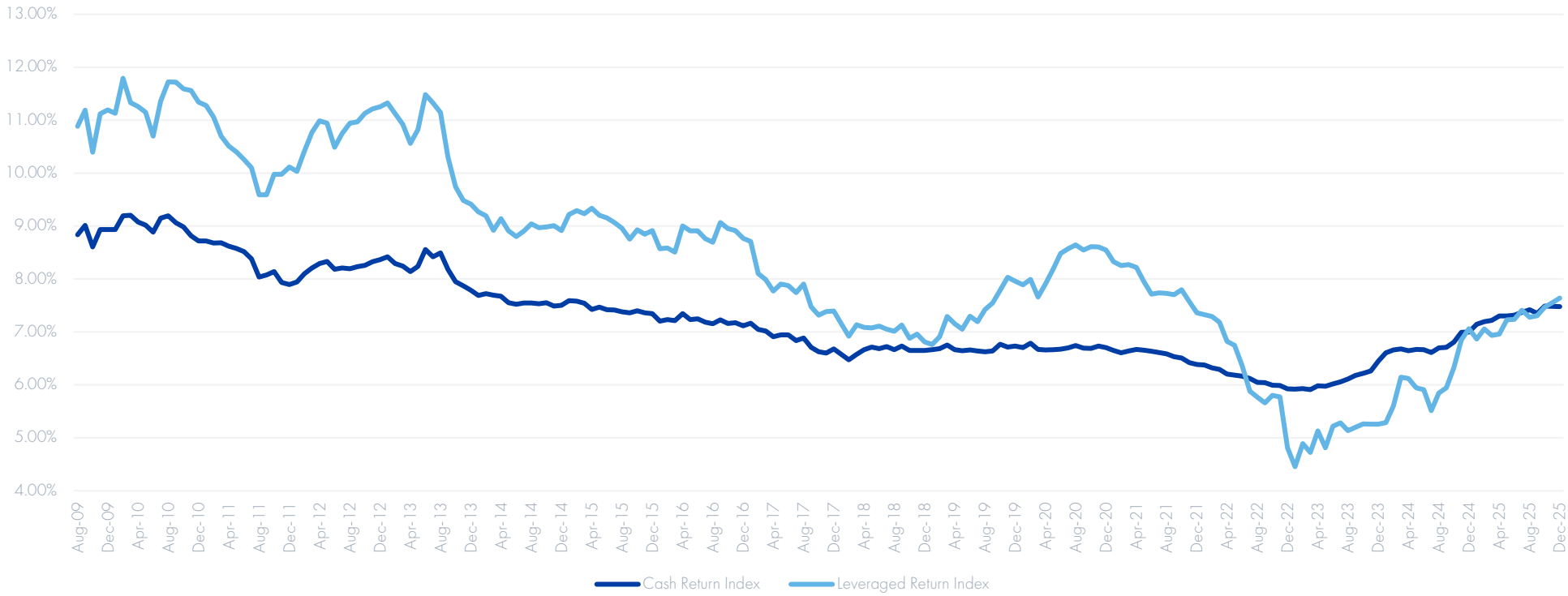
10-yr UST: 4.12% on Dec 31, 2025; year-end data confirms cap-rate stabilization with gradual easing contingent on financing conditions and asset quality

Dialysis Capital Markets

ANNUALIZED CASH AND LEVERAGED RETURN

What the chart shows: This panel compares an all-cash return proxy for dialysis NNN (dark line) with a modeled cash-on-cash "leveraged return" (light line). The cash return index blends trailing-twelve-month average cap rates (with upper/lower quartiles) over ~15 years; the leveraged index assumes 50% leverage, 30-year amortization, and a note rate set at ~10-year UST + 180–220 bps at the time of acquisition, applied to the average cash cap rate. The goal is to visualize how financing conditions have influenced investor yield through cycles.

2025 year-end read-through: With the 10-year UST ending the year at ~**4.12%**—down from 4.24% at mid-year and 4.39% at year-end 2024—financing costs eased modestly, and the modeled leveraged return (**7.51%**) is now tracking above the cash return index (**7.40%**) for the first time in several quarters. This positive leverage crossover, even if narrow, signals that accretive financing is increasingly available for well-located, longer-term dialysis assets when coupons print inside the lower end of our assumed spread band. Use this panel alongside the prior Valuation Index and the next few pages to calibrate expectations: the path to wider positive leverage depends on continued rate easing and selective lender appetite.



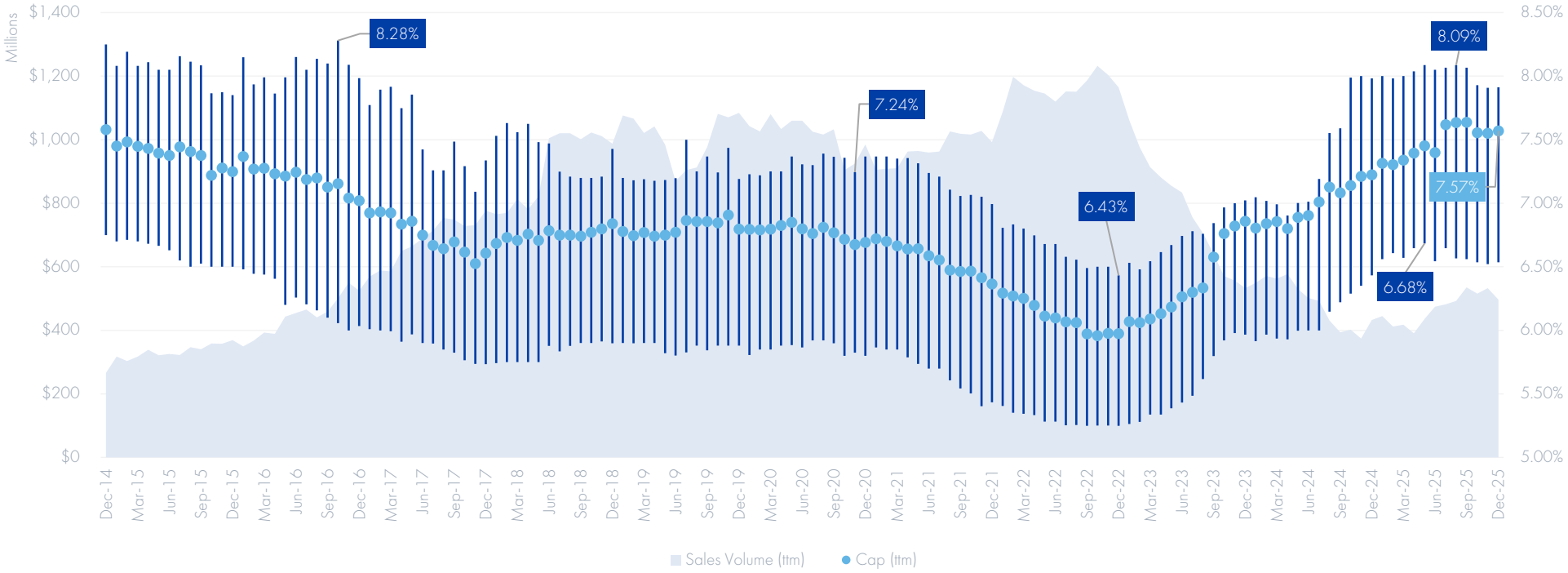
10-yr UST 4.12% (12/31/25); net-lease cap-rate trend = stabilizing; modeled leverage = 50% LTV, 30-yr am, 10Y+180–220 bps.

Dialysis Capital Markets

VOLUME & CAP RATE RANGES

Where we are now (year-end 2025): Following the 2022–24 repricing, the second half of 2025 confirmed stabilization and the beginning of a volume recovery: trailing-twelve-month transaction count reached **152** (up from 138 at mid-year and 118 a year ago), and TTM sales volume rose to **$496.7 million—a +11.6% year-over-year increase**, marking the first positive YoY print since late 2021. The 10-year U.S. Treasury ended 2025 at ~**4.12%**, down 27 bps from mid-year, providing a steadier underwriting backdrop. Within single-tenant dialysis, the TTM average cap rate printed **7.57%** with an upper quartile of **7.91%** and lower quartile of **6.54%**. Core, longer-term deals continue to clear well inside the broader market average.

How to read this page: The blue shaded area tracks trailing-twelve-month (TTM) sales volume; the dot marks the TTM average cap rate; and the band shows upper/lower quartile cap rates. Use this alongside our active-listings snapshot to calibrate expectations: as of Q4 2025, marketed inventory stands at **108 listings** (vs. 97 in Q2 2025 and 107 a year ago), while average asking caps sit at **6.54%** for the total market and **5.70%** for core 10+-year offerings. These trends are consistent with broader outpatient momentum (health systems continue to push care to MOB/outpatient settings), which supports durable demand for dialysis real estate even as capital remains rate-sensitive.



Year-end 2025: volume recovery confirmed (+11.6% YoY); cap rates stabilizing near cycle highs; core dialysis trades remain inside the overall market.

Dialysis Capital Markets

TREND WATCH (through December 31, 2025)

Rates eased into year-end.

The 10-Year U.S. Treasury finished 4Q25 at ~4.12%, down from 4.24% at mid-year and 4.39% at year-end 2024—offering a modestly more constructive financing backdrop.

Volume inflection confirmed.

TTM transaction count reached 152 with $496.7M in volume, up 11.6% year-over-year—the first positive annual volume change since late 2021. All three buyer categories (individual, fund, REIT) expanded meaningfully versus mid-year.

Cap rates plateaued; selective compression emerging.

The TTM average cap rate printed 7.57%, marginally above mid-year's 7.40%, but the pace of expansion decelerated to just +25 bps (annualized) versus +39 bps at mid-year—consistent with a plateau rather than another leg higher. Core 12+ year assets traded at 6.89% on average.

Healthcare/MOB demand remains constructive.

Providers continue shifting care to outpatient settings; MOB absorption and investor interest in medical-adjacent corridors remain strong, reinforcing durable demand for the dialysis clinic footprint.

Operator fundamentals: stable.

FMC reiterated its 2025 outlook with U.S. same-market treatment growth >0.5%; DaVita reported steady utilization through Q2 2025. These datapoints support steady occupancy assumptions heading into 2026.

Policy watch—Medicare ESRD PPS.

CMS proposed lifting the ESRD base rate for CY2026 to $281.06 (+~1.9% vs. CY2025's $273.82), a modest tailwind to clinic revenue that investors will monitor into 2026 underwriting.

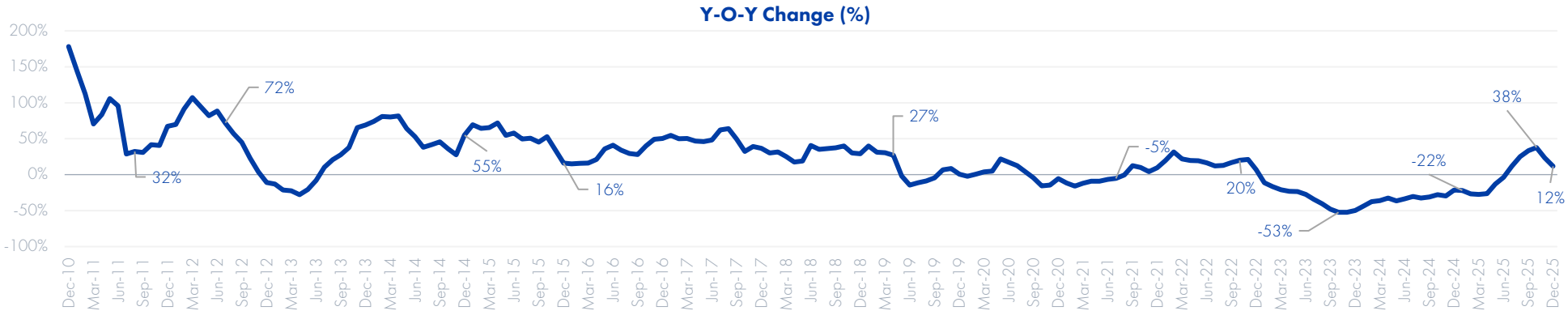
What this means for dialysis pricing in 2026: Expect stable-to-modestly-firming headline cap rates with continued dispersion by asset quality: newer clinics, long primary terms, and meaningful rent bumps should command the tightest caps, while shorter-term or above-market-rent deals may still require concessions. The combination of lower rates, improving volume, and broadening buyer participation sets the stage for a more liquid market in 2026—particularly if the 10-year remains below 4.25%.

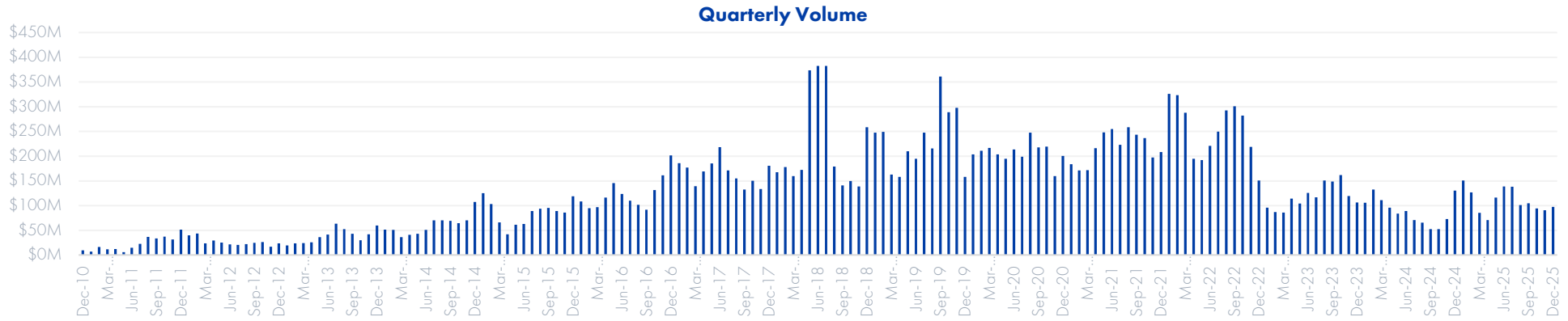
Dialysis Capital Markets

TRANSACTION VELOCITY AND ALLOCATION TO SINGLE-TENANT DIALYSIS

Allocation across cycles (what the first chart shows): Our tracking over ~15 years shows that sharp pullbacks in allocation to single-tenant dialysis typically coincide with periods of rising rates—with recovery beginning as volatility cools and underwriting visibility improves. By year-end 2025, the recovery was firmly underway: U.S. net-lease investment volume turned positive on a trailing basis, and TTM dialysis sales volume reached $496.7 million (+11.6% YoY)—the first positive annual comparison since late 2021. Year-over-year allocation of **+12%** through year-end compares favorably to the –22% and –53% reads of the prior two years.

Quarterly volume (how to read the second chart): The bars show quarterly sales counts/volume in the dialysis subset. The 2025 cadence accelerated meaningfully from mid-year forward, with the TTM transaction count rising to **152** (from 118 at year-end 2024 and a trough of 103 in late 2024). With the 10-yr UST at ~4.12% on December 31, 2025 (down 27 bps from mid-year), buyers are re-engaging more broadly: individual buyers rose to **68** (from 49 at mid-year), fund buyers to **56** (from 42), and REITs to **11** (from 6). Track this page with the cap-rate and listings panels that follow to calibrate bid/ask.



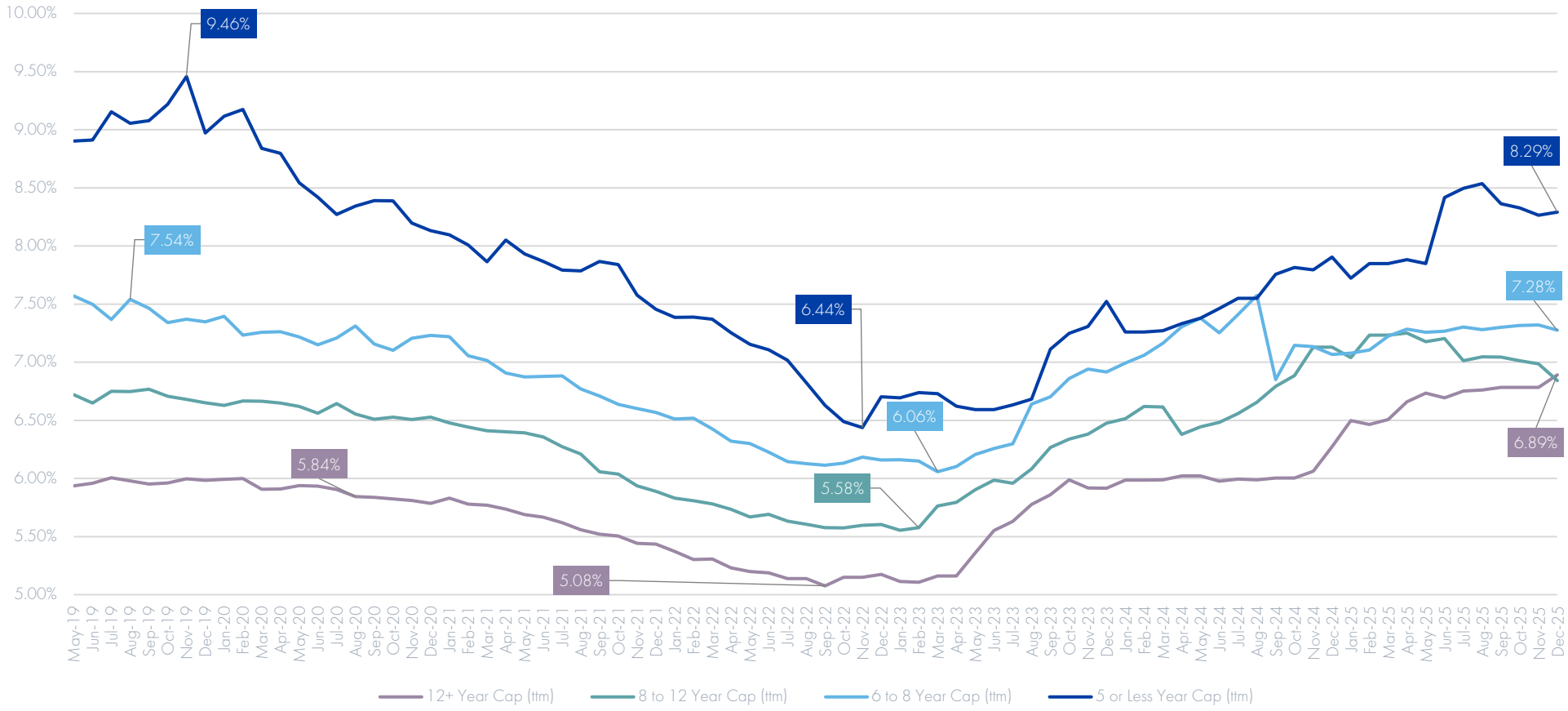


Dialysis Capital Markets

CAP RATE COMPARISON: THE VALUE OF LEASE TERM

How to read this page: The purple line tracks the TTM average cap rate for dialysis sales with >12 years of primary term remaining; green = 8–12 years; sky blue = 6–8 years; and blue = ≤5 years. The intent is to isolate the term premium investors assign to longer lease term, all else equal.

Year-end 2025 read-through: With the 10-year UST ending 2025 at ~4.12%, financing costs eased modestly, supporting tighter pricing for longer-term, higher-quality leases. The 12+ year cohort averaged **6.89%** (up from 6.69% at mid-year), while the 8–12 year cohort compressed to **6.84%** (down from 7.20%)—a notable convergence that may reflect thin sample sizes or specific deal characteristics in the mid-term bucket. The 6–8 year cohort held at **7.28%**, and the ≤5 year bucket ticked down to **8.29%** (from 8.42%). The term premium between the longest and shortest cohorts stands at ~**140 bps**, consistent with a still-meaningful reward for lease term even as the overall cap rate environment stabilizes. Use this page in tandem with the adjacent rate and volume panels when calibrating bids.



Dialysis Capital Markets

COST OF CAPITAL AND THE IMPACT INTEREST RATES HAVE ON VALUATION

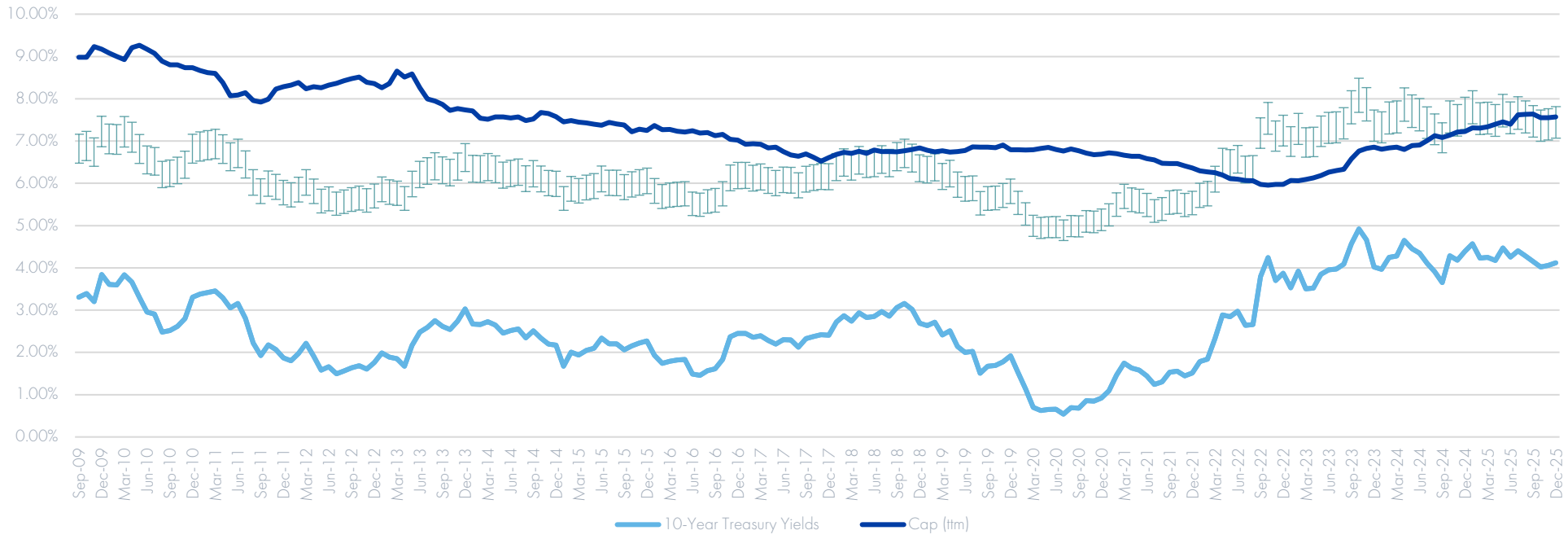
Why rates still set the tone: Cap rate is the market's clearing point between available dialysis assets and active buyers, and one of the biggest demand-side inputs is the availability and cost of debt. Most lenders price to a spread over the 10-Year U.S. Treasury, so movements in the 10-year are an early read-through to buyer yields and, by extension, clearing cap rates. The chart visualizes this relationship over time for the dialysis subset. As of December 31, 2025, the 10-year finished at ~**4.12%**; under the assumed structure (10Y + 180–220 bps; 30-yr amortization), that implies an illustrative loan-constant band in the low-to-mid-7%s (≈**6.98%–7.30%**). With cap rates largely plateauing after the 2022–24 run-up and the TTM average at **7.57%**, the band now sits meaningfully below the average clearing cap—supporting **positive leverage** on a broader set of deals than at mid-year, when the loan-constant band was ≈7.23%–7.54%.

How to read this page:

Sky-blue line: the 10-Year UST over time.

Blue line: the TTM average cap rate for dialysis sales.

Green band: an assumed mortgage constant built from a 30-year amortization with loan coupon = 10Y + 180–220 bps; use it to gauge positive (cap > constant), neutral (cap ≈ constant), or negative (cap < constant) leverage conditions.



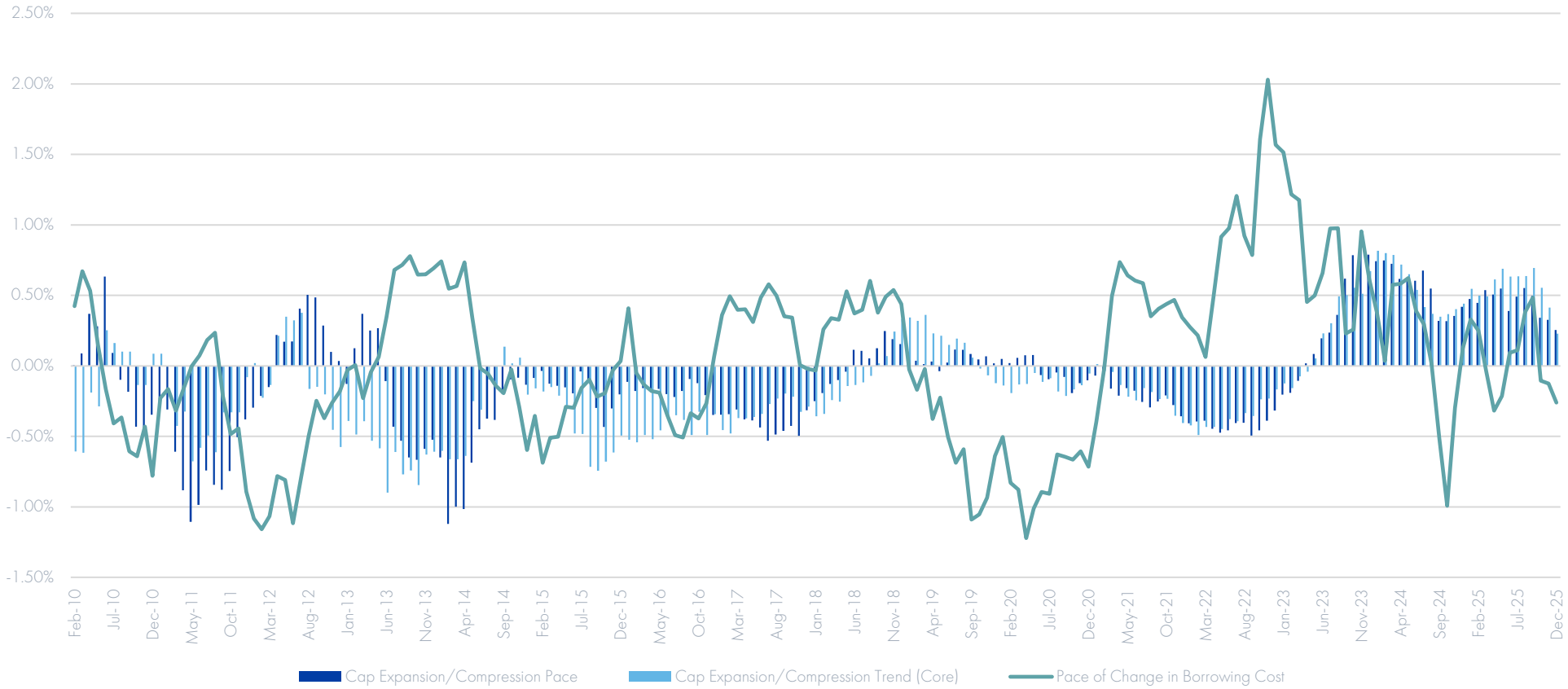
Year-end 2025: rates easing; net-lease cap rates stabilizing; positive leverage achievable on a broader set of deals. Loan-constant band based on 10Y + 180–220 bps, 30-yr am.

Dialysis Capital Markets

PACE OF CAP RATE EXPANSION

What changed in 4Q25: After the 2022–24 reset, the quarter-over-quarter pace of cap rate expansion has decelerated meaningfully. Our Q4 2025 reading shows the annualized expansion pace at just **+25 bps**—down from +39 bps at mid-year and +42 bps at year-end 2024—evidence that the repricing cycle is largely complete. At the same time, the 10-Year U.S. Treasury ended Q4 at ~**4.12%** (December 31, 2025), down 13 bps from mid-year and 27 bps from year-end 2024, supporting a narrower bid-ask environment.

How to read this page: This chart tracks the quarterly delta (basis points) in our dialysis cap-rate series. Use the recent bars to benchmark today's move versus prior spikes: (i) declining bars through 2H25 confirm deceleration toward zero, (ii) dispersion by term/credit/market quality remains wide (strong, long-term clinics can clear well inside the average at sub-7%), and (iii) with rates easing rather than flat, we expect the expansion pace to approach zero or flip negative in 1H26 for core assets. Keep this page in context with the prior Cost of Capital panel and the upcoming listings snapshot when discerning the path forward for 2026.



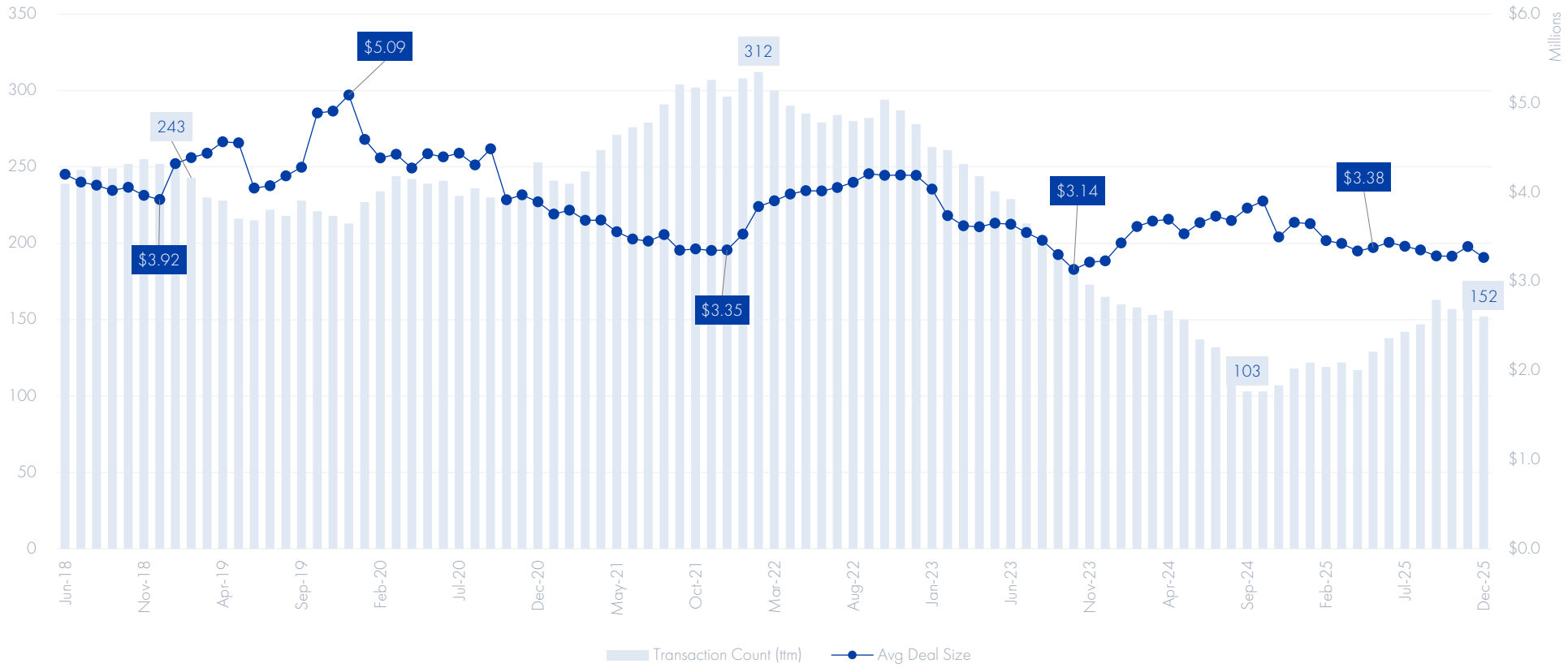
Dialysis Capital Markets

TRANSACTION COUNT & AVERAGE DEAL SIZE

Where we are now (through 4Q25): Activity recovered meaningfully in the second half of 2025: the trailing-twelve-month (TTM) transaction count reached **152**, up from 138 at mid-year and well above the late-2024 trough of **103**. The five-year average remains ~248, so current volumes are still below cycle norms—but the trajectory is clearly upward. Recall that the cycle high was 312 sales (TTM) in February 2022.

Average deal size settled to **$3.27M** (from $3.44M at mid-year and $3.66M a year ago), with dispersion driven by lease term, rent steps, and build-out scope. Elevated construction and site costs (retail-adjacent, medical-grade interiors) still influence pricing, partly offset by retrofit opportunities where appropriate.

How to read this page: The bars show the TTM transaction count; the line tracks TTM average deal size. The 2022 high (312 TTM) and the late-2024 trough (103) frame a 2025 environment that turned the corner—volume is up +11.6% year-over-year and buyer participation broadened across all three categories (individual, fund, REIT). Pair this with the neighboring rate and cap-rate pages to calibrate bid/ask and size expectations for 2026.

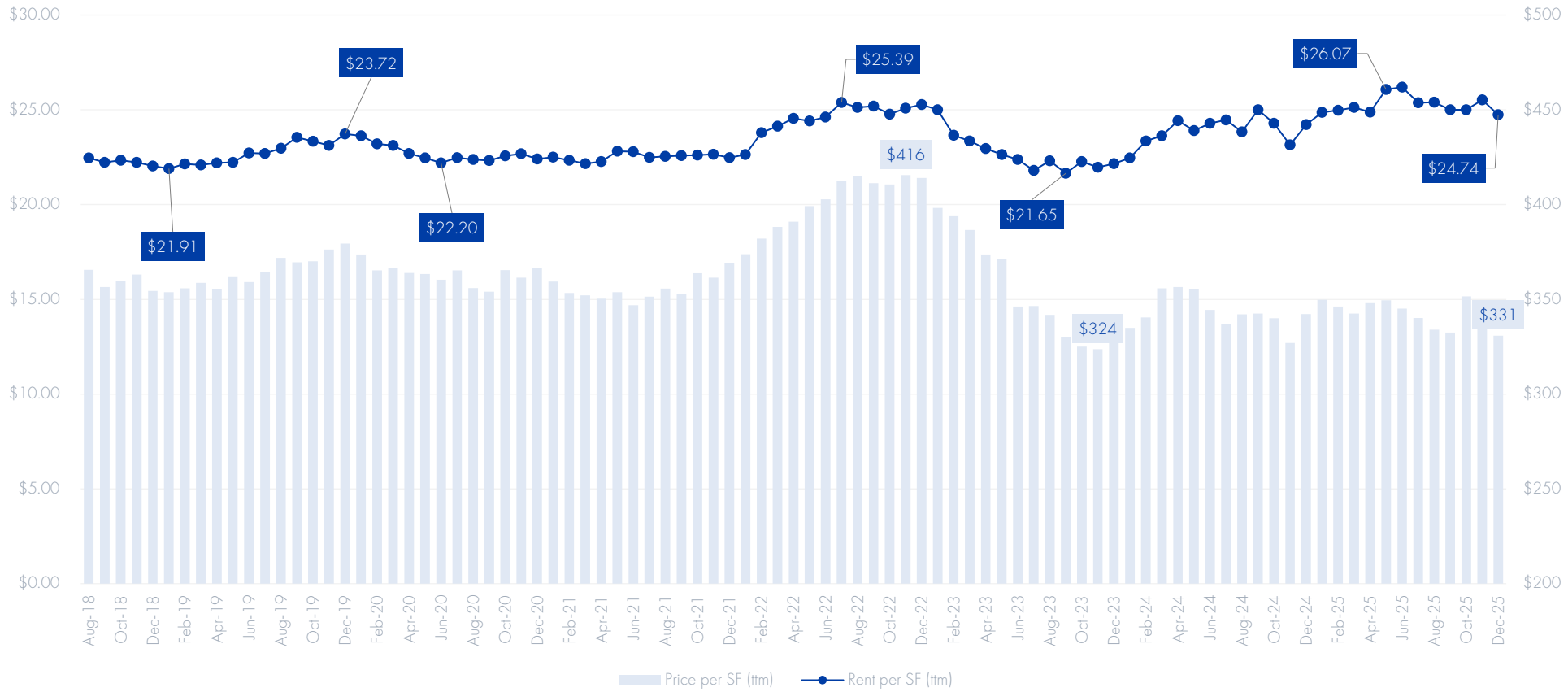


Dialysis Capital Markets

RENT & PRICE PER SQUARE FOOT

What the chart shows (through 4Q25): The panel tracks TTM average rent ($/SF) and TTM average price ($/SF) for closed single-tenant dialysis sales. The long-run averages remain roughly $22/SF rent and $323/SF price, with current readings at **$24.74/SF** rent and **$330.83/SF** price—both above baselines given higher medical build-out specs and preferred, retail-adjacent locations. Rent/SF moderated from $26.20 at mid-year, while price/SF eased from $345.08, reflecting mix shifts in the deal pool. Expect wide deal-by-deal dispersion reflecting term, credit, site quality, and interior scope.

How to use it: Read the two series together to understand whether today's price/SF and rent/SF are moving in tandem in your submarket. Elevated construction and land costs continue to support higher asking rents; where retrofits are feasible, they can temper delivered basis and help keep economics in range.



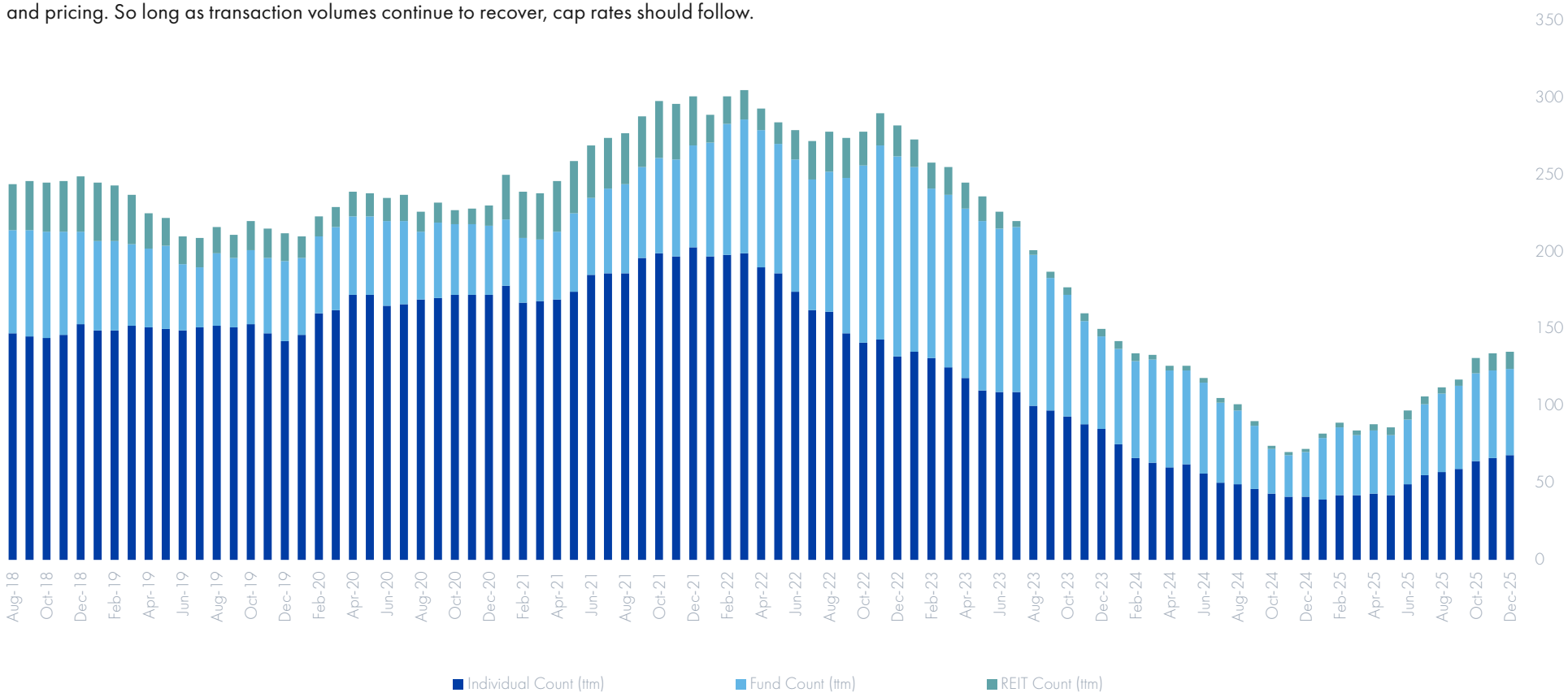
Dialysis Capital Markets

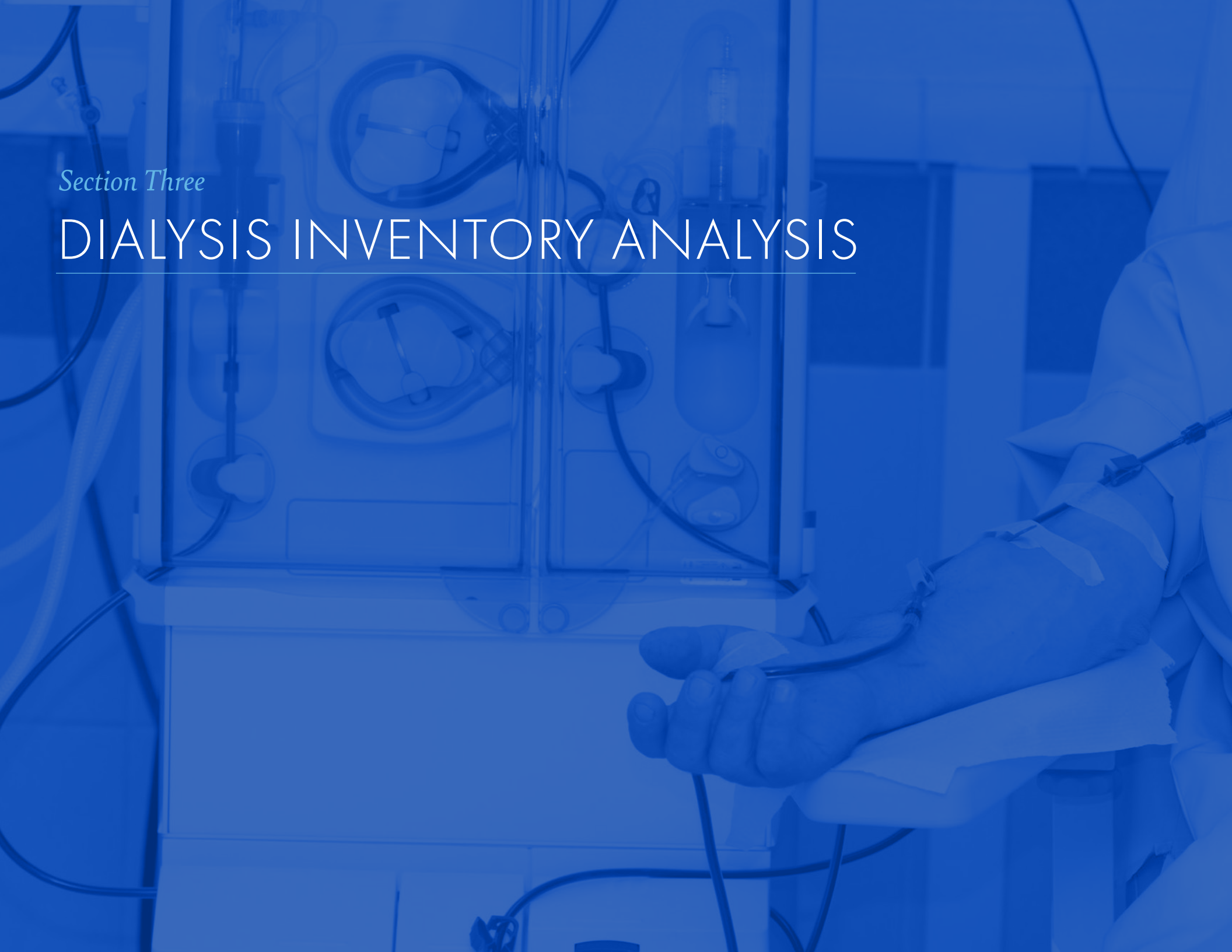
BUYER POOL & CAP RATES

Investor demand for quality single-tenant office, industrial and medical product such as a dialysis clinic is recovering from the 2022–24 repricing cycle, with all three buyer categories expanding through year-end 2025. The cost of capital eased modestly as the 10-year UST declined to 4.12% by December 31, 2025, encouraging re-engagement—particularly among private and exchange-driven investors.

Overall, the private or individual buyer remains the largest portion of the buyer pool. This is largely driven by the tax-deferred exchange investor. Sales create gains, gains create exchange needs, exchange needs and shortened investment timelines drive competition and pricing. So long as transaction volumes continue to recover, cap rates should follow.

The chart below depicts a composition by capital structure of the buyer pool by transaction count of single-tenant dialysis-leased sales. The Individual buyer category includes family offices, individuals or trusts and it expanded to 68 transactions (TTM) from 49 at mid-year. The Fund buyer category includes any aggregation of private capital, syndicator or specialized investment vehicle, reaching 56 transactions (from 42 at mid-year). The public listed/REITs buyer category expanded to 11 transactions (from 6 at mid-year)—still below the long-term average of 21 but trending in the right direction as public capital returns to the space.





Section Three

DIALYSIS INVENTORY ANALYSIS

Dialysis Inventory Analysis

ON-MARKET SNAPSHOT

Total Market (all deals on market): Listed inventory edged up to **108 clinics** (from 97 at mid-year and 107 a year ago). Average asking price stands at **$3,718,644**, while the average asking cap sits at **6.54%** (down from 6.67% at mid-year). Dispersion remains wide: upper quartile **7.00%** (down from 7.25%), lower quartile **6.00%** (roughly flat), with a median **6.50%** (vs. 6.45%). Average days on market (calculated) stands at **401 days**, and **42%** of listings recorded a price change—a slight improvement from mid-year's 53%, suggesting sellers are increasingly pricing to market at the outset.

Core "10+ Year Term": Supply remains lean with **14 offerings** (stable versus mid-year's 14, down from 31 a year ago). Average price sits at **$4,832,238**, with average asking cap **5.70%** (down from 6.19% at mid-year—a meaningful move). Quartiles remain tighter than the broader market (upper **6.04%**, lower **5.24%**). Median prints **5.70%**. Price changes were logged on 57% of core listings—elevated, reflecting the longer marketing durations typical of higher-priced, longer-term assets.

How to use this page: This snapshot pairs counts with asking cap metrics to visualize where sellers are pricing and how buyer selectivity shows up in time on market and price adjustments. Read it alongside the following "Supply-Side Metrics" pages for trend context across multiple quarters.

TOTAL MARKET	CURRENT Q4 2025		Q4 2024
NUMBER AVAILABLE	108	↑	107
AVERAGE PRICE	$3,718,644	↑	$3,693,854
AVERAGE CAP	6.54%	↓	6.75%
UPPER QUARTILE	7.00%	↓	7.25%
LOWER QUARTILE	6.00%	↑	5.90%
MEDIAN	6.50%	↑	6.45%
DAYS ON MARKET	401	↑	297
PRICE CHANGE	42%	↓	60%

10+ YEAR TERM	CURRENT Q4 2025		Q4 2024
NUMBER AVAILABLE	14	↓	16
AVERAGE PRICE	$4,832,238	↓	$5,192,854
AVERAGE CAP	5.70%	↓	6.13%
UPPER QUARTILE	6.04%	↓	6.40%
LOWER QUARTILE	5.24%	↓	5.74%
MEDIAN	5.70%	↓	6.02%
DAYS ON MARKET	642	↑	333
PRICE CHANGE	57%	↑	50%

Dialysis Inventory Analysis

SUPPLY SIDE METRICS

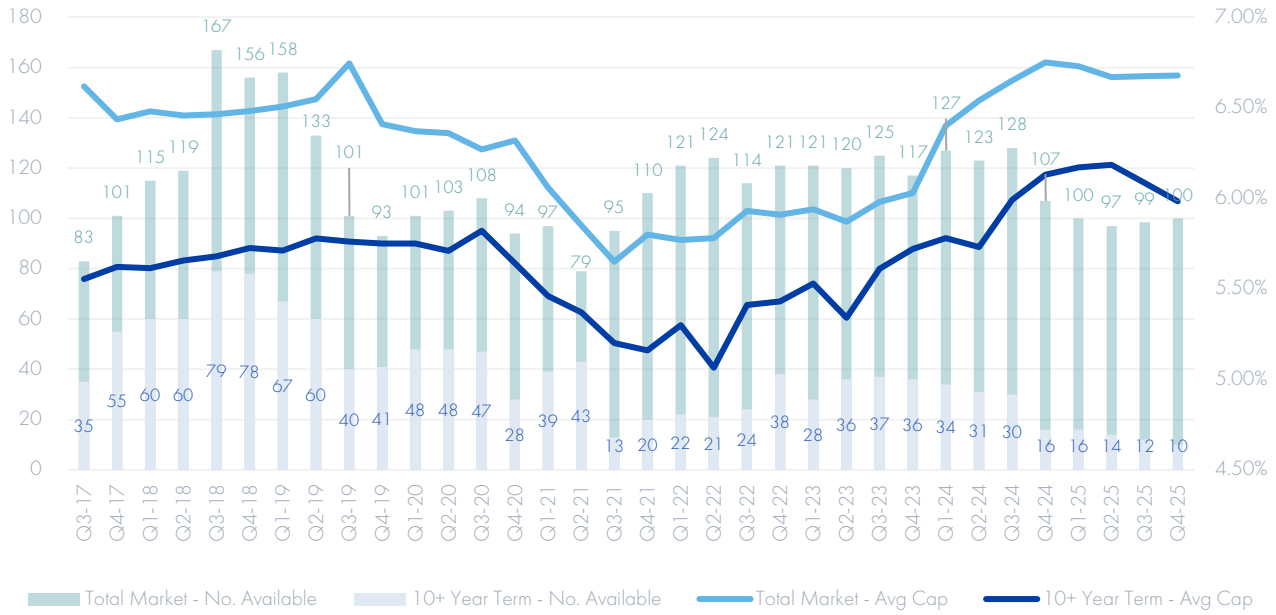
The charts that follow are an attempt to graphically depict the supply side of this cap rate equation both as a snapshot in time over the current reported period but also over time.

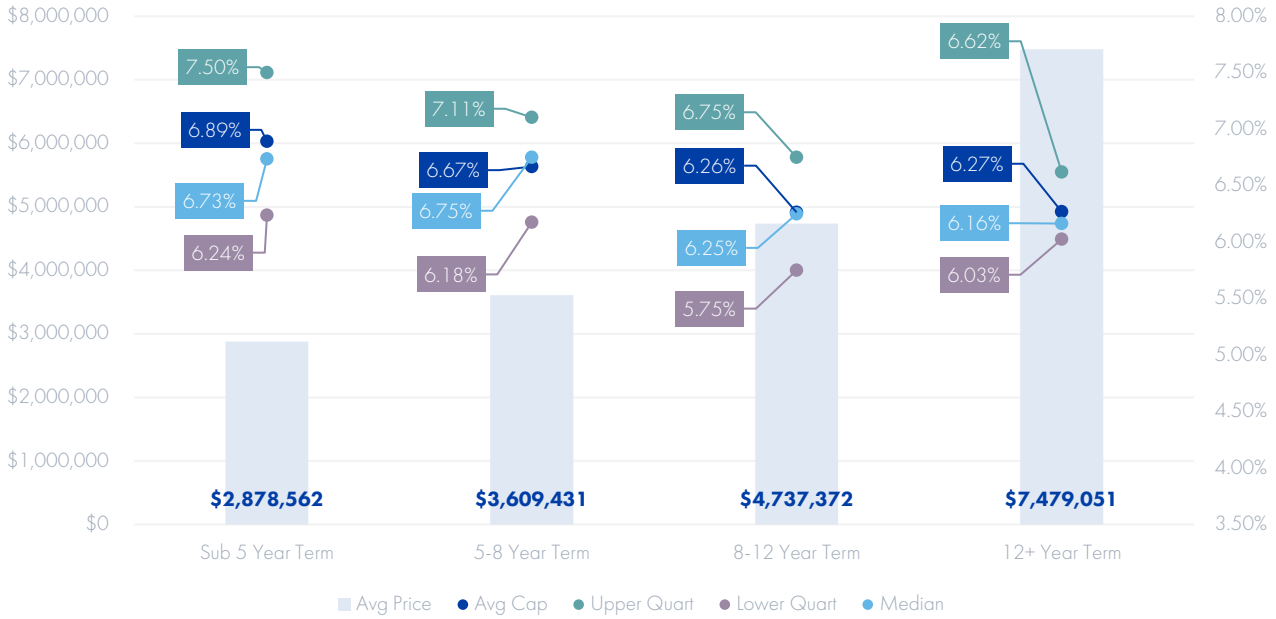
AVAILABLE MARKET SIZE

What the top chart shows (over time): We track the number of clinics actively marketed (bars) and the average asking cap rate (lines) for both the Total Market and the Core (10+ years remaining) subset. Through 4Q25, listed supply is stable while core pricing tightened: 108 total clinics vs. 97 at mid-year (107 a year ago); 14 core vs. 14 at mid-year (16 a year ago). The current average asking cap prints 6.54% for the Total Market (down from 6.67%) and 5.70% for Core 10+ years (down from 6.19%).

ASKING CAP RATE RANGES

What the bottom chart shows (quartiles): We plot upper/lower quartile asking caps over time to visualize seller sentiment and where pricing is heading. As of the latest cut: Core lower quartile **5.25%** (long-run avg 5.24%) and upper quartile **6.47%** (long-run avg 5.88%). Total Market lower quartile **5.80%** (long-run avg 5.46%) and upper quartile **7.25%** (long-run avg 6.78%). Use these bands to gauge where your specific asset (term, credit, location, rent steps) should clear relative to the market.



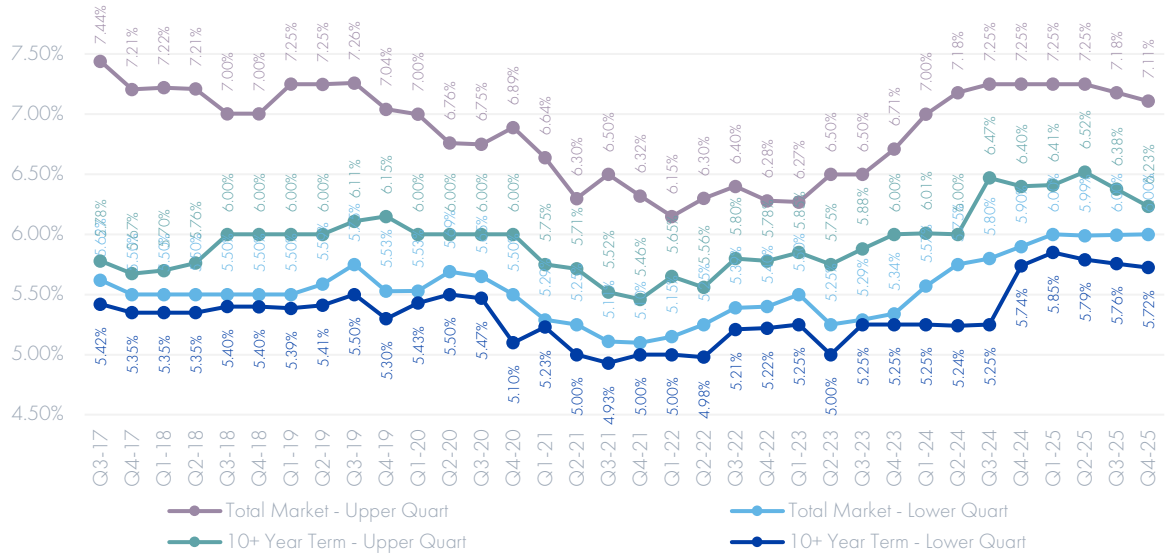


Dialysis Inventory Analysis

AVAILABLE MARKET SIZE

ASKING CAP RATE RANGES

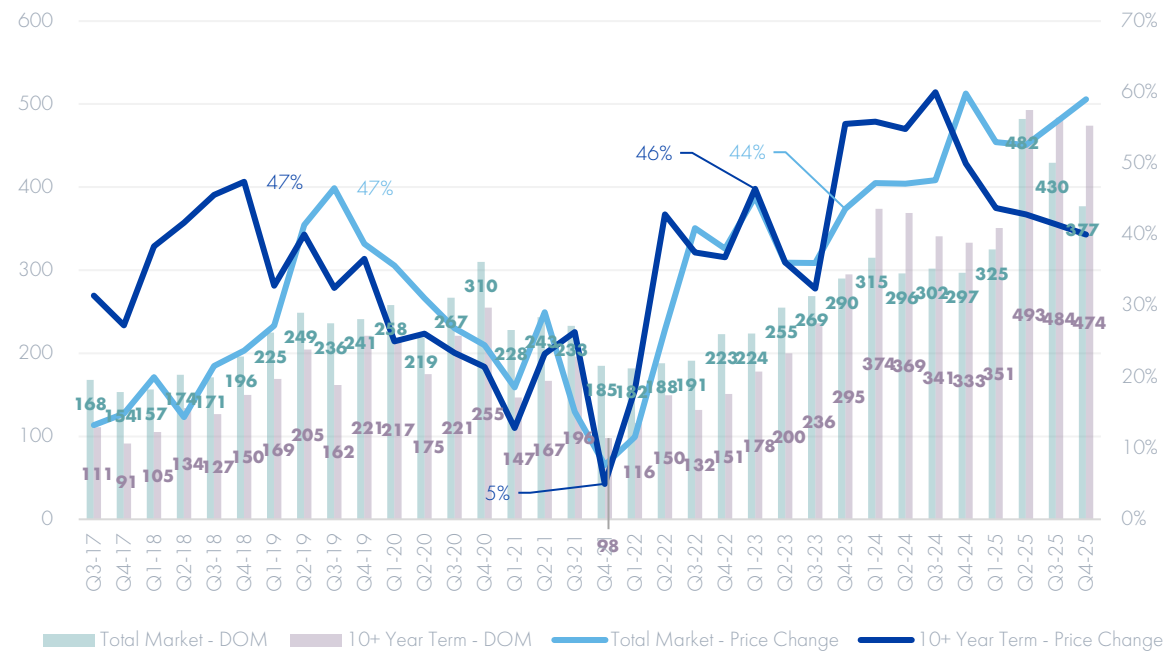
The chart at right shows a summary of all dialysis-leased investment properties actively being marketed at various points over time. It is broken down by "core" dialysis leased sales (10+ years primary lease term rem.), and the total number of clinics available for sale (including the 10-year+ deals). It depicts the upper and lower quartile asking cap rate ranges and tracks this trend over time. This is an illustration of seller sentiment and serves as a good guidepost for where cap rates are and are heading. The current core lower quartile is **6.00%** while its long-term average is 5.72%. The current core upper quartile is **7.11%** while its long-term average is a 6.23%. The overall lower quartile is a **5.99%** while its long-term average is a 5.46%. The overall upper quartile is a **7.25%** while its long-term average is a 6.78%.



MARKETING DURATION & PRICE ADJUSTMENTS

What the chart shows (through 4Q25): The left panel tracks average days on market (DOM) for active listings—Total Market vs. Core (10+ years remaining). The right panel tracks the share of active listings with at least one price change over the period shown. As of Q4 2025, DOM remains elevated versus a year ago: Total Market = **401 days** (vs. 297 in Q4 2024) and Core = **642 days** (vs. 333). This reflects a selective bid environment even as activity re-engages.

How to use this page: Pair DOM with price-change frequency to gauge seller/buyer alignment: in Q4 2025, **42%** of Total Market listings had a price change (vs. 60% in Q4 2024), while Core 10+ Year listings showed **57%** with a change (up from 50%), consistent with scarce core supply and tighter pricing discipline on the broader market side. Read these alongside your On-Market Snapshot and Available Market Size pages to calibrate go-to-market timing and initial pricing strategy.



Dialysis Inventory Analysis

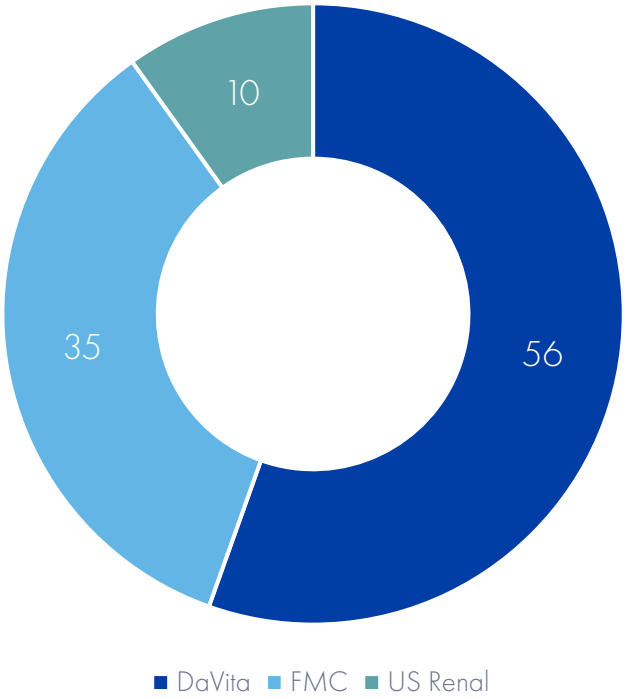
AVAILABLE CLINICS BY TENANT

What the table shows: The snapshot breaks out active listings by major operator, with count, aggregate asking volume, average deal size, average primary term remaining, average asking cap, and average days on market.

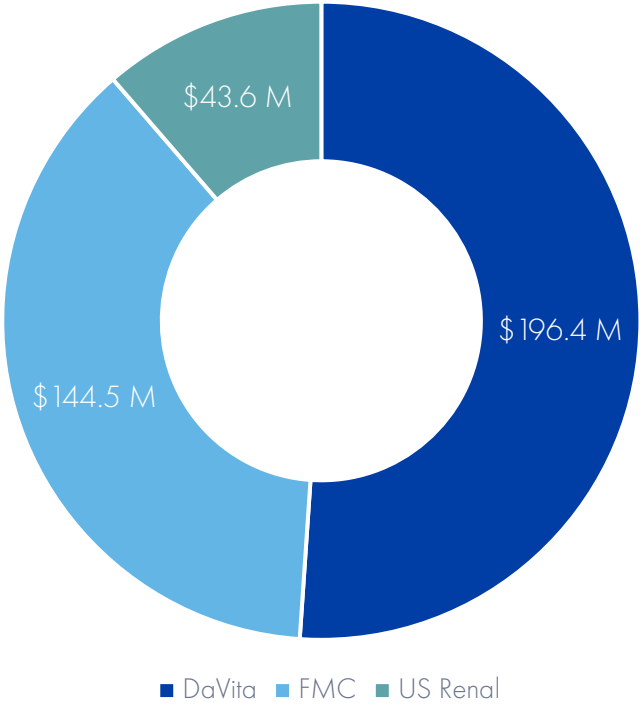
How to use this page: Read mix (who's marketing) alongside pricing and timing: DaVita currently represents the largest share of available clinics by count and volume, while U.S. Renal's smaller sample skews to a higher average price and slightly lower asking cap than peers—consistent with longer build-outs or longer terms on select offerings. Differences in DOM (DaVita > FMC in this cut) can reflect asset quality, lease term, and pricing discipline; treat these as directional, not prescriptive, and pair with the prior On-Market Snapshot and the next DOM & % of Ask page for context

	COUNT AVAILABLE	VOLUME AVAILABLE	AVG. DEAL SIZE	AVG. TERM REMAINING	AVG. ASKING CAP	AVG. DAYS ON MARKET
DaVita	56	$196,421,319	$3,507,524	6.10	6.57%	454
FMC	35	$144,543,265	$4,129,808	6.44	6.36%	444
US Renal	10	$43,565,892	$4,356,589	4.93	6.65%	427

COUNT AVAILABLE



VOLUME AVAILABLE



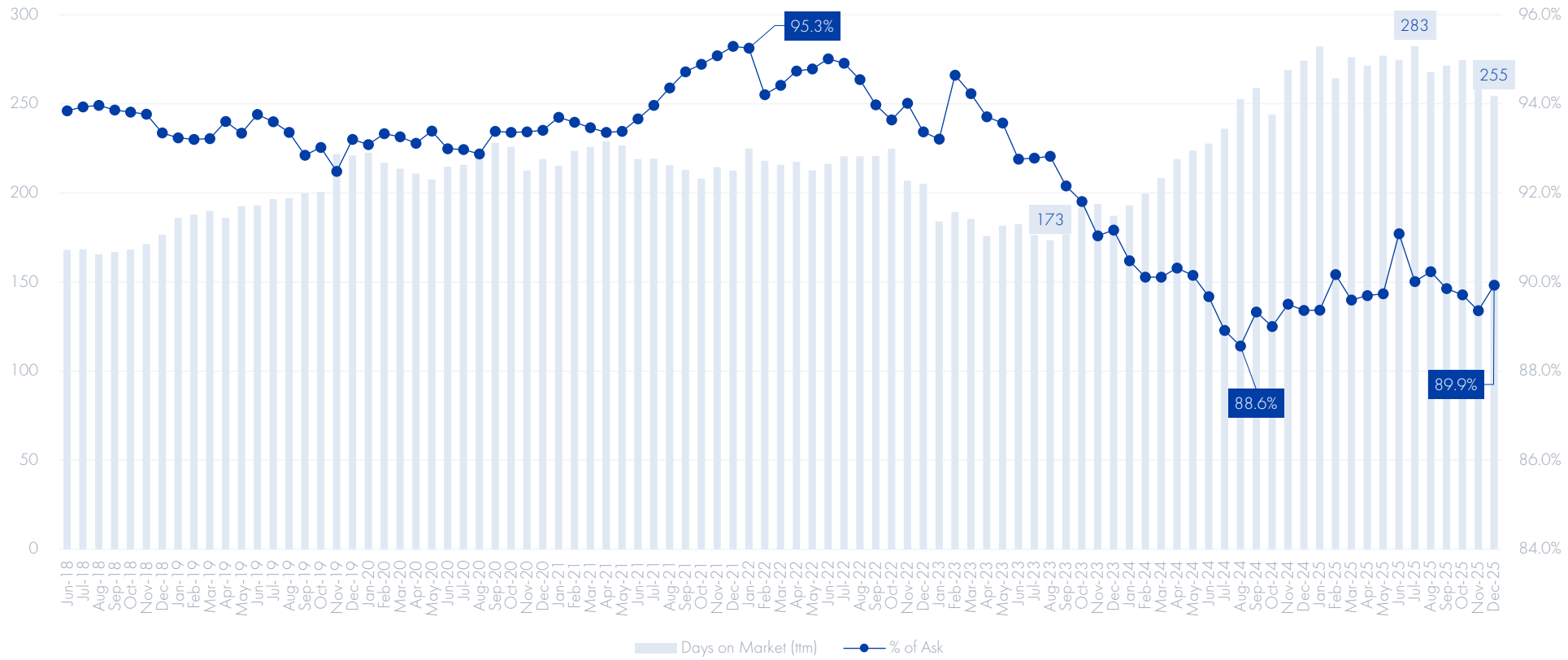
Dialysis Inventory Analysis

DAYS ON MARKET & PERCENTAGE OF ASK PRICE

What the chart shows (through 4Q25): The left axis tracks the average time on market (days) for clinics that actually sold in the trailing twelve months—from first broadly marketed date through closing. The right axis tracks the average sales price achieved as a % of asking price for those broadly marketed sales over the same TTM window. Keep the long-term benchmarks in view: the historical DOM average is ~173 days, and the historical % of ask average is ~93.7%.

As of year-end 2025, DOM for closed deals averaged **255** days (down from 275 at mid-year), while % of ask stands at **89.9%** (down from 91.1% at mid-year). The DOM improvement is a positive signal—assets that are correctly priced are moving faster—while the lower % of ask reflects the ongoing recalibration of seller expectations to today's rate environment.

How to use it (context for 2026): Pair this page with the adjacent On-Market and Marketing Duration & Price Adjustments panels: DOM is improving for closed deals even as active listings show longer shelf lives, which indicates bifurcation—well-priced assets clear; stale inventory lingers.

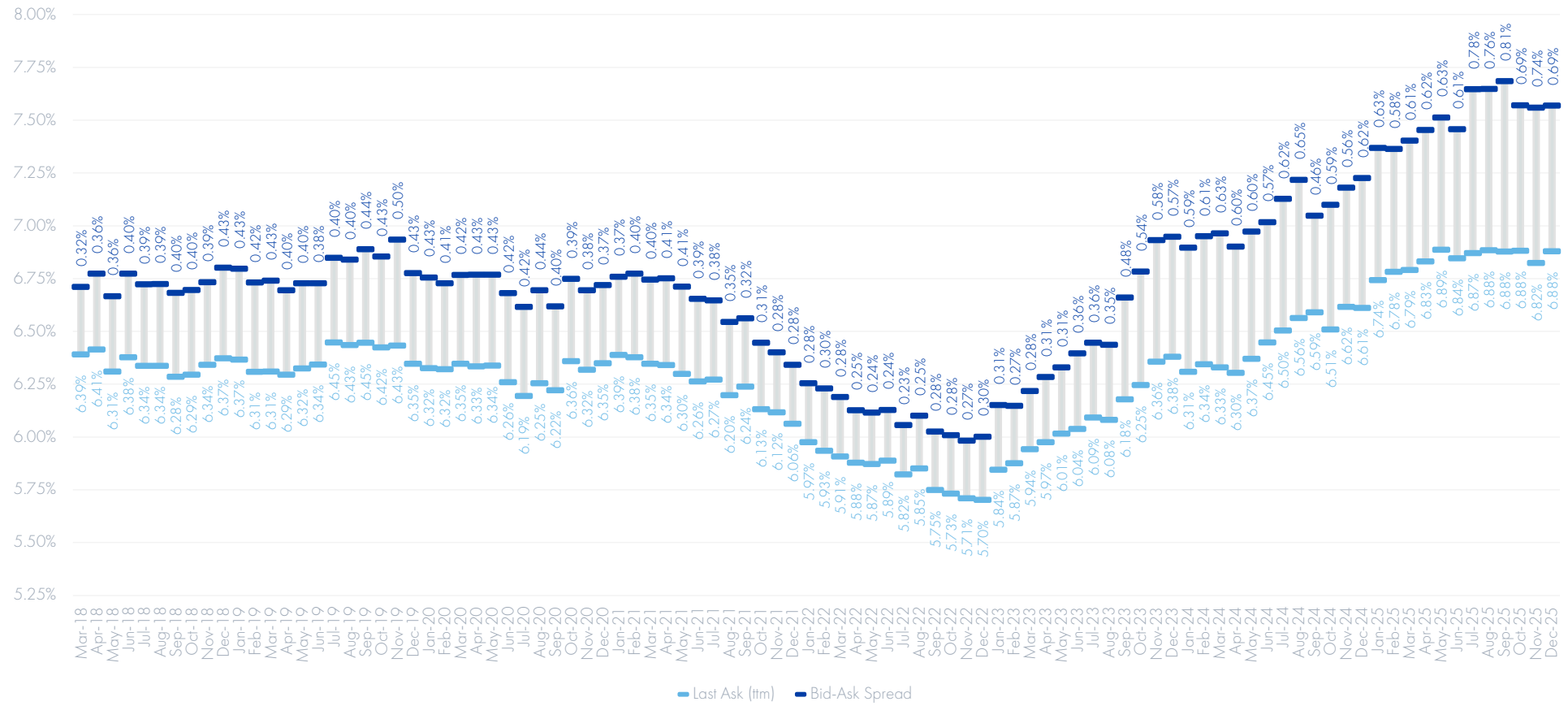


Dialysis Inventory Analysis

BID-ASK SPREAD

What the chart shows: The sky-blue section at the base of each high-low bar is the last broadly marketed asking cap rate for clinics that ultimately sold in the trailing 12 months; the full bar shows the range of last asks for those TTM sales. Use this series to spot turning points when short-run "last asks" diverge from the longer-run view.

Deal-level bid-ask spread (how to read it at year-end 2025): The line plots the average per-deal spread between each sale's last ask and its achieved cap—not the difference between TTM averages. The long-run average spread in this dataset is ~44 bps; the most recent reading stands at **69 bps**, widened from 61 bps at mid-year. This widening reflects a mix of assets clearing in the market rather than a re-acceleration of repricing: as volume recovered (+11.6% YoY), more deals transacted—some with larger pricing concessions—pulling the average spread wider even as headline cap rates stabilized. The 10-year UST ended the year at ~4.12%, and the last-ask average has risen to **6.88%** (from 6.84% at mid-year), indicating sellers are pricing closer to market but buyers are still extracting concessions on execution.

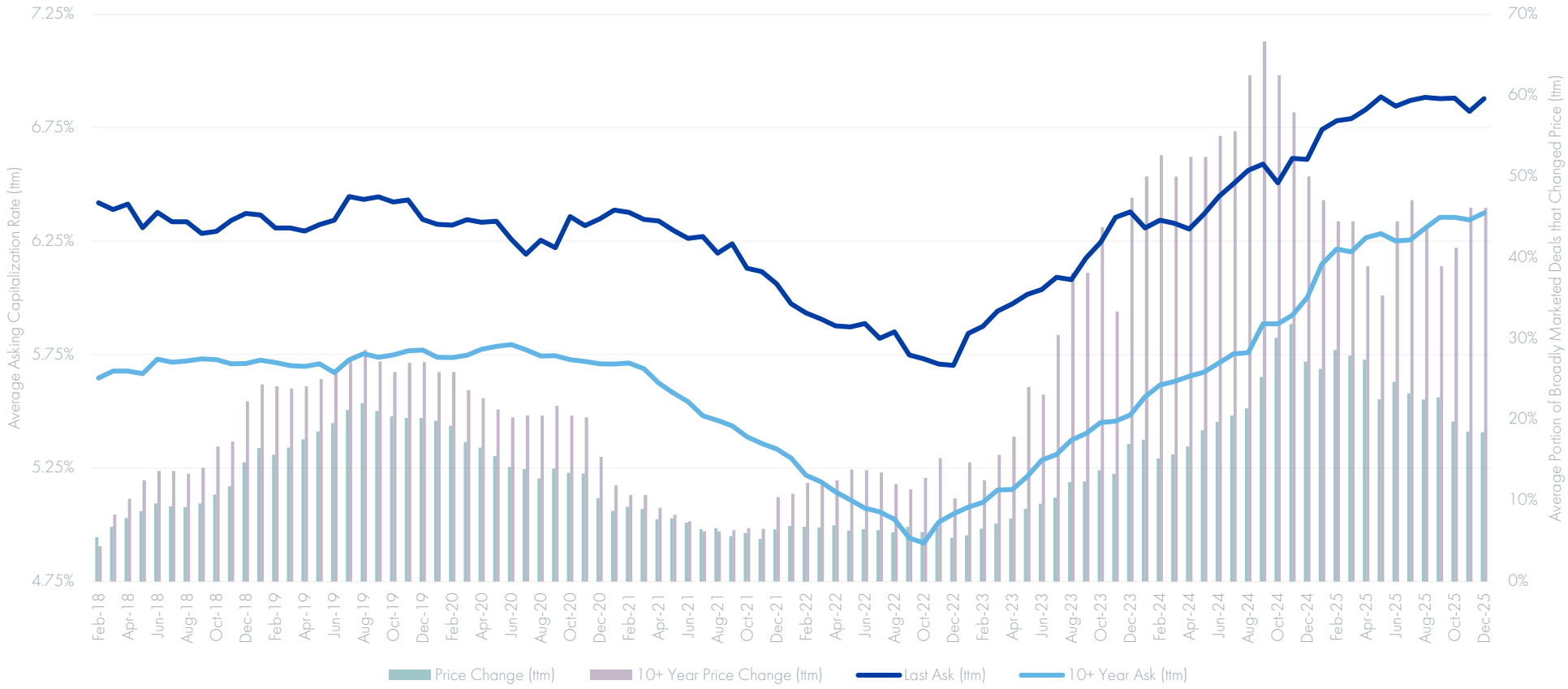


Dialysis Inventory Analysis

SUPPLY SIDE: SELLER SENTIMENT AND CONFIDENCE

What the chart shows: The bars track the share of broadly marketed deals that closed after a price adjustment before closing: green = all dialysis, purple = 10+ years remaining at close. The blue/sky-blue overlays are the last asking cap rates for those same cohorts just prior to close. Use this page as a supply-side complement to your demand/rates views.

How to read the latest bars (through 4Q25): The most recent cut shows price-change incidence at **18.4%** for all deals (down from 24.6% at mid-year) and **46.2%** for 10+ year core deals (up from 44.4%). The decline in overall price-change frequency signals improving seller pricing discipline—more deals are listing at or near clearing levels from the outset, consistent with an 18-month learning curve as the market adjusts to post-2022 debt costs. Core (10+ yr) offerings still see elevated pre-close adjustments given their longer marketing periods and higher price points. Net: supply-side pricing is finding equilibrium, with the 10-year UST at 4.12% providing an incrementally more supportive backdrop heading into 2026.



Section Four

ABOUT NORTHMARQ

Yielding Results

The principal catalyst in Northmarq's Commercial Investment Sales' (CIS) success over its competitors is the combination of our platform and our concentrated focus on net lease. These two differentiators allow our team to realize extra value for our Sellers by leveraging our position as the **bridge between private and institutional capital** in our sales process.

COLLABORATION

Northmarq CIS operates on an open, collaborative platform in which we share information and one database among all the firm's brokers. By leveraging our collective knowledge and strengths, we are able to maximize value and exposure for our clients – a thorough and complete representation of the market, unmatched by our competitors.

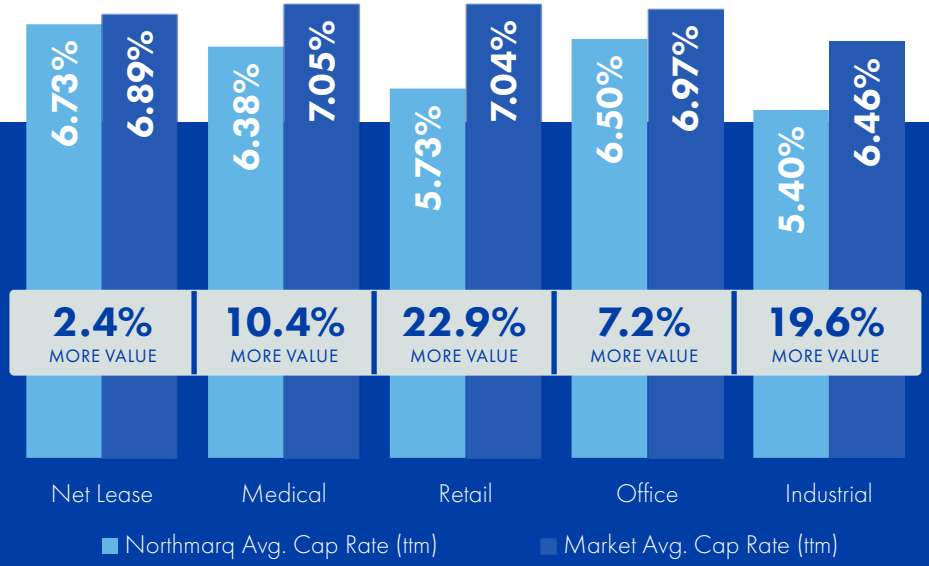
SPECIALIZATION

Northmarq Commercial Investment Sales focuses on net lease sales and maintains the largest team of focused net lease brokers in the country. As a result, Northmarq is viewed as the preeminent source of net lease knowledge and properties by buyers, fellow brokers and influencers across the country.

Northmarq's Commercial Investment Sales consistently monitors all net lease investor activity through a proprietary central database system—including advanced analytics such as website downloads and visits—shared by all Northmarq professionals. Our national network of professionals track and share the activity of all active investors historically, in the market and those considering a net lease investment including buyers, sellers, REITs, institutions, private equity, foreign capital, lenders, tax deferred investors (1031, 1033), high net worth private investors, and family offices.

AVERAGE CAP RATE | NORTHMARQ VS. OTHER BROKERS

Our track record demonstrates that our process produces superior results. The data below depicts the results our approach yields and solidifies our status at the peak of net lease investment sales. Our competition may know the top fifty or so repeat buyers and owners of net lease assets nationwide but our platform allows a reach beyond the known owners and active investors. Over the past twelve months, Northmarq brokered the sale of over $4.74 billion of net lease properties nationwide – approximately ***$2.06 billion (or 43%) of these sales were acquired by a tax-deferred exchange investor.***



*TTM Q4 2024 Results as Reported by Real Capital Analytics

Value Proposition Results

NET LEASE DIALYSIS

The result speaks for itself. The below is a graphical depiction of the average capitalization rate for all brokered net lease dialysis transactions sold over the previous twelve months. The sky blue line represents the average capitalization rate on net lease dialysis sales brokered by Northmarq brokers. As of December 31, 2025, Northmarq brokered sales averaged a **6.70%** capitalization rate. The gray line represents the average capitalization rate on net lease dialysis sales brokered by all other brokers over that same time period. As of December 31, 2025, non-Northmarq brokered sales average a **7.33%** capitalization rate. In other words, Northmarq's proprietary sales process and marketing approach yielded the average dialysis Seller **$265,788** in additional proceeds or **10% more value** over the past twelve months when compared with all other brokers.

AVERAGE DIALYSIS-LEASED NOI

$205,141

AVERAGE DIALYSIS CAP RATE (TTM)

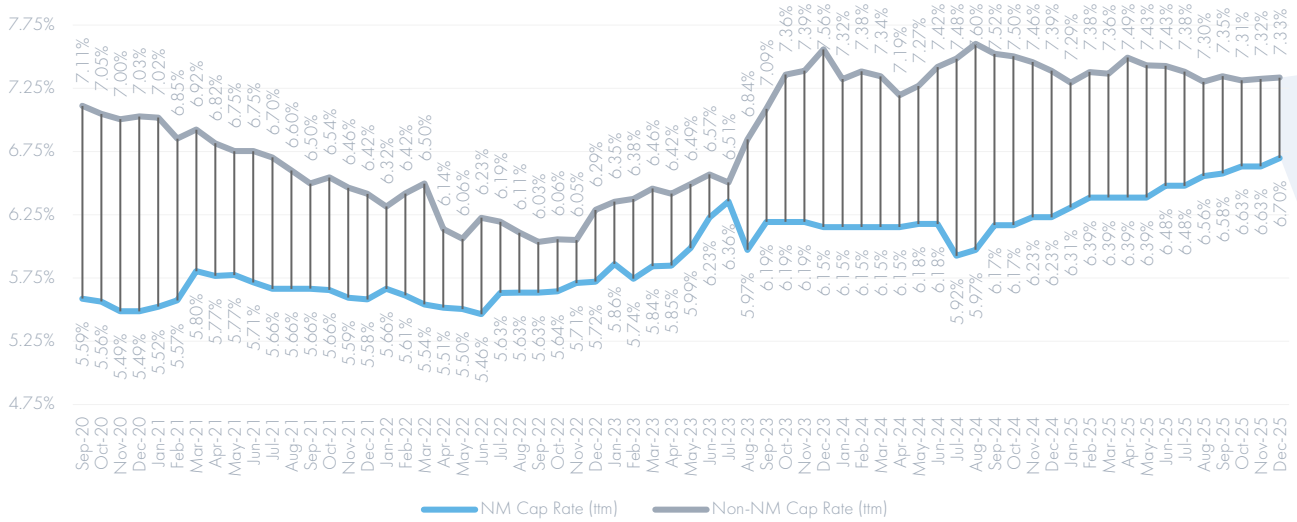
6.70%
Northmarq

7.33%
Non-Northmarq

AVERAGE DIALYSIS SALES PRICE

$3,062,996
Northmarq

$2,797,207
Non-Northmarq



10%
Additional Value Realized by Northmarq

$265,788
Additional Proceeds Realized by Northmarq

Healthcare Track Record

A vital component of Northmarq's success has been a dedicated focus and specialization by its professionals in a certain product category such as healthcare transactions. Our single-tenant healthcare experience includes more than 310 healthcare transactions totaling more than $1.37 billion in value over the previous five years.

- **Diversified client representation of both buyers and sellers**
- **Exclusive listings and off-market transactions**
- **Access to tax-deferred exchange buyers**
- **Strong relationships with repeat, non-trade driven clients**
- **Joint-venture and structured capital solution advisory**

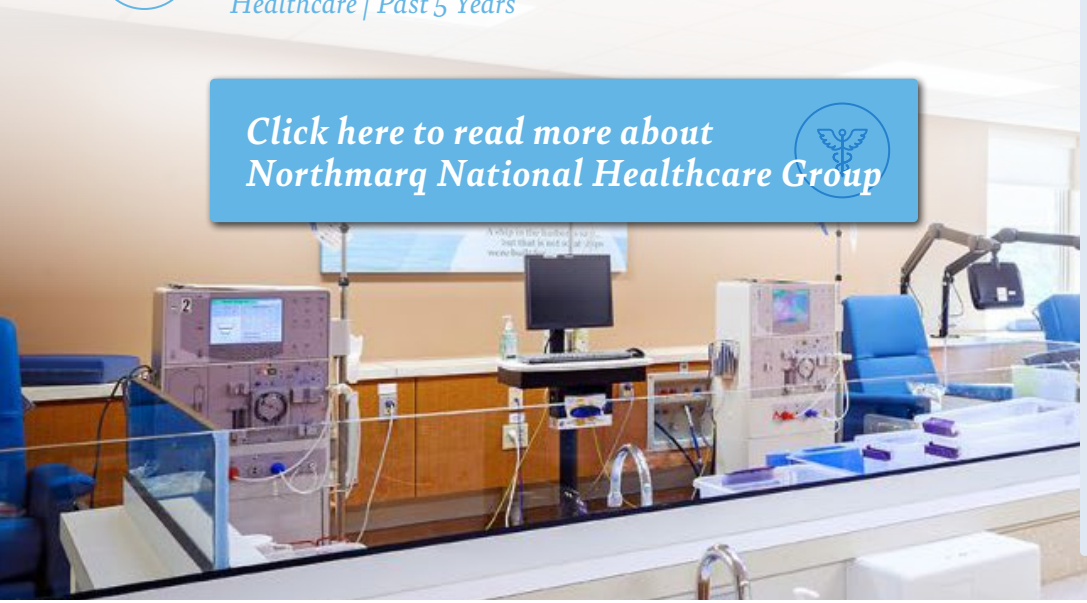
110+ Clinics Sold

Dialysis | Past 5 Years

$1.37B Sales Volume

Healthcare | Past 5 Years

Click here to read more about Northmarq National Healthcare Group



NOTABLE HEALTHCARE TRANSACTIONS

TENANT	SALE PRICE
VA Medical Center	$160,000,000
DaVita, Inc. HQ 2	$93,527,390
Tulsa Bone & Joint	$72,500,000
Vibra Rehabilitation Hospital	$69,000,000
Arkansas Surgical Specialty Hospital	$56,600,000
Colorado General Hospital	$51,000,000
MOB Portfolio of 6	$46,500,000
Dignity Health/Emerus Hospital Portfolio of 2	$46,280,700
Summit Medical Group MOB	$45,000,000
Magan Medical Clinic	$39,456,600
BluePearl Veterinary Portfolio of 6	$39,456,000
VA Outpatient Clinic	$33,307,000
Lakeview Health	$31,500,000
Los Angeles Military Entrance Processing Station	$31,140,000
PRA Health Sciences	$31,100,000
Greater Peoria Specialty Hospital	$30,200,000
Christus Mother Frances Hospital	$29,870,000
VA Outpatient Clinic	$29,472,000
Cobalt Rehabilitation Hospital	$28,473,300
Fox Valley Hematology & Oncology	$28,200,000
The Orthopedic & Sports Institute of the Fox Valley	$27,900,000
Stanislas Surgical Hospital	$26,700,000
Reliant Hospital	$24,400,000
Abbott Diabetes Care	$23,925,000
Cobalt Rehabilitational Hospital	$23,660,000
Dignity Health/Emerus Hospital	$23,029,300
Emerus Hospital Zaramora	$22,506,000

Sample Dialysis Transactions

DaVita Dialysis MOB

KANSAS CITY, MO

$8,700,000 | 8.88% | PRIVATE CAPITAL

DaVita Dialysis

MURFREESBORO, TN

$5,575,505 | 5.20% | 1031 EXCHANGE

DaVita Dialysis

WOODLAND, WA

$4,837,997 | 6.25% | PRIVATE CAPITAL

DaVita Dialysis

SANTA MARIA, CA

$8,800,000 | 4.61% | 1031 EXCHANGE

Fresenius Medical Care

AUSTINTOWN, OH

$3,365,058 | 6.30% | 1031 EXCHANGE

U.S. Renal Care

MISSION, TX

$2,380,000 | 6.50% | 1031 EXCHANGE

DaVita Dialysis

SANFORD, FL

$3,356,928 | 5.00% | 1031 EXCHANGE

Targee Dialysis, LLC

NORTH COLLEGE HILL, OH

$3,271,879 | 5.30% | 1031 EXCHANGE

DaVita Dialysis

LAWRENCEVILLE, GA

$4,779,607 | 5.35% | 1031 EXCHANGE

DaVita Dialysis

OAKLAND, CA

$14,275,000 | 4.76% | PRIVATE CAPITAL

Fresenius Medical Care

SAN ANTONIO, TX

$4,236,893 | 5.14% | 1031 EXCHANGE

Fresenius Medical Care

COLORADO SPRINGS, CO

$4,900,000 | 4.94% | 1031 EXCHANGE

Services Offered

FORMAL MARKETING

The value of every single-tenant transaction is directly affected by the investor audience exposed to and bidding on the opportunity. Exclusively engage Northmarq to initiate formal marketing of your asset and your property will receive broad, systematic exposure to an unrivaled audience of single-tenant investors worldwide. This will drive competition among buyers and will drive at maximizing proceeds all while assuring seamless execution. Others may boast about a one-off sale but our unmatched approach consistently yields market making results with our time-tested ability to qualify offers.

PRIVATE PLACEMENT

Leverage the collective knowledge, expertise and track record of Northmarq to discretely, privately and directly place an opportunity with a proven Buyer at a competitively-generated price that will provide for sure and timely execution and an abbreviated marketing stint.

PERIODIC VALUATION

The market is ever-changing. Pervasive shifts and dynamic trends are inescapable for the real estate investor. Periodic valuation and an on-demand assessment of marketability can aide in determining when to monetize and conclude an investment. Our team provides annual valuation for our clients for just this reason. Take advantage of our market expertise.

MARKET INTELLIGENCE AND ANALYSIS

A significant focus of what we do is single-tenant dialysis-leased. Take advantage of our market intelligence - comparable sales, market data, rental rates, sample lease terms, marketing feedback, etc.

TAX-DEFERRED EXCHANGES

Our team coordinates the acquisition of replacement property in the single-tenant market for our clients on tax-deferred exchanges. We have handled multiple large and complex exchanges including one as large as $160 million in cumulative value that represented eleven separate transactions. Avoid the stress and allow us to partner with you through this stressful period, holding your hand through various complexities that may arise. Our reach is unmatched by our competition, engage our team and see higher quality on and off market product when searching for replacement product.

BTS/JV EQUITY PLACEMENT

Knowledgeable brokers generate competition in the buyer pool to drive at maximizing sales proceeds. Generate competition when searching for a capital source to help fund future projects in order to free up resources and diversify the risk in the development pipeline. Friends and family or institutional partners are not the only routes to pursue when funding a build-to-suit development. Northmarq has successfully closed on many equity sourcing or joint-venture transactions that represent over $1.2 billion in total constructed value, which includes more than $125 million in equity capital raised.

FULL-SERVICE CAPITAL SOLUTIONS/FORWARD COMMITMENT

Our team has significant experience with structured transactions. Forward commitment or loan-to-own type structures can be a strong hedge against potential future market risks when working to bring a new build-to-suit development online.

Northmarq Debt + Equity

Commercial real estate transaction financing through trusted, long-term relationships

As the largest privately owned provider of commercial real estate financing in the country, Northmarq helps clients capitalize on real estate opportunities through our unmatched network of commercial real estate lender partners.

We have established strong, long-term relationships with institutional providers of debt and equity capital, including life insurance companies, Freddie Mac, Fannie Mae, FHA/HUD, Wall Street and local, regional and national banks.

We value the trust our clients place in us, and we are committed to providing balanced guidance and appropriate solutions for your unique commercial real estate financing needs.

COMMERCIAL PROPERTY FINANCING — DEBT

When you need financing for your commercial or multifamily property, you need a trusted adviser who can put all of your best commercial mortgage options on the table. Northmarq has long-term relationships with a variety of proven lenders, ensuring the best solution for your commercial real estate financing needs. Upon closing, work with our Loan Servicing team and receive unparalleled support throughout the life of your loan.

No matter what your commercial real estate debt financing needs are—a first mortgage, bridge financing, construction financing, affordable housing or tax-exempt loans, or a complex mix—we can help.

COMMERCIAL PROPERTY FINANCING — EQUITY

Sometimes the best financing option for your commercial property involves finding equity partners. Benefit from Northmarq's expansive network of investor relationships and market knowledge to structure the right capital stack for your commercial real estate investment.

When you want to invest in commercial real estate, Northmarq has the experience and the breadth of options to be your trusted adviser for any equity investment.

Built with your success in mind, we offer a strong foundation to help you thrive with commercial real estate financing opportunities for every property. For an innovative solution to any commercial mortgage financing challenge, contact **John Heitmann**, Vice President, Debt + Equity, National Healthcare Group.

John Heitmann
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Debt + Equity Financing

- Acquisition
- Bridge
- Refinance
- Equity
- Mezzanine

Leverage our debt financing relationships to find your real estate capital

Life Insurance Companies

Freddie Mac

Fannie Mae

HUD/FHA

CMBS

Banks

Acronym Glossary

ACRONYM	DEFINITION
AAKH	Advancing American Kidney Health (2019 Executive Order initiative)
CKD	Chronic Kidney Disease (stages 1–5)
CKF/CKCC	Comprehensive Kidney Care First / Comprehensive Kidney Care Contracting (CMS voluntary models)
DVA	DaVita, Inc. (NYSE: DVA)
ESRD/ESKD	End-Stage Renal Disease / End-Stage Kidney Disease (interchangeable; CMS uses ESRD, clinical literature increasingly uses ESKD)
ETC	ESRD Treatment Choices (CMS mandatory payment model)
FMC/FMS	Fresenius Medical Care (FWB: FME; NYSE ADR: FMS)
GLP-1 RA	Glucagon-Like Peptide-1 Receptor Agonist (drug class including semaglutide/Ozempic)
HD	Hemodialysis
IOTA	Increasing Organ Transplant Access (CMS mandatory model, go-live July 2025)
KCC	Kidney Care Choices (CMS value-based model umbrella)
MEDPAC	Medicare Payment Advisory Commission
NHE	National Health Expenditures
NNN	Triple Net (lease structure)
OPTN	Organ Procurement and Transplantation Network
PD	Peritoneal Dialysis
PPS	Prospective Payment System (CMS bundled payment for ESRD)
SRTR	Scientific Registry of Transplant Recipients
USRDS	United States Renal Data System

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Commercial Real Estate
Debt + Equity | Investment Sales | Loan Servicing | Fund Management

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